uniQure N.V. (Nasdaq: QURE)

Company Update: FY2026E–FY2033E Revenue and EBITDA Forecast and Finagle Valuation

2 November 2026. US$ millions unless a row states patients or a count. Includes a Finagle valuation (Section 8) but carries no rating or price target. Numbered sources are in Appendix B; the forecast and valuation files are listed there [D45][D46].

About this report

The company. uniQure N.V. (QURE) is a Netherlands-based gene-therapy company listed on Nasdaq. Today almost all of its revenue is royalties on Hemgenix, a haemophilia B gene therapy licensed to CSL Behring [A21]. The single material driver of its value is AMT-130, a one-time gene therapy for Huntington's disease (HD), delivered by MRI-guided neurosurgery using ClearPoint Neuro's system. uniQure filed for US approval (a BLA) and UK approval (an MAA) in September 2026 [G12]. On 2 November 2026 the FDA accepted the BLA under the Accelerated Approval pathway with Priority Review and a target action date (PDUFA) of 1 May 2027, and said it tentatively plans an advisory committee meeting [G701]. If AMT-130 is approved, it would dwarf the royalty business; if it is not, uniQure is a small royalty company with a large cost base.

What the report is. It is a revenue and EBITDA forecast for FY2026E–FY2033E in three scenarios: **base** (approval on 1 July 2027, our central assumptions), **bull** (approval, faster capacity, higher price and larger demand) and **bear** (no approval). The forecast is then passed to Finagle, an external valuation model, which extends it to FY2035E and computes a value per share for each scenario. The report gives no rating or price target.

How it was built, step by step.

1. **History and guidance.** Reported years stay as reported (10-K FY2025 [D1]). FY2026E is analyst consensus, because the company gives no revenue guidance.
2. **Scenario assumptions.** For each scenario we set approval timing, capacity, price, demand and cost assumptions (Appendix A gives bull and bear).
3. **Volumes from an eligible-patient pool.** AMT-130 is a one-time treatment for patients in a narrow, early stage of HD. So patient numbers come from a "pool" of eligible patients: it starts at uniQure's estimate of ~6,000 at launch, is refilled each year by newly diagnosed patients, and is drained by treatment and by patients who progress out of eligibility or die (Appendix D).
4. **Capacity limits.** Each patient needs specialised neurosurgery, so in the early years the number treated is limited by how many procedures centres can do, not by demand. We use a ClearPoint capacity ramp of 330 / 2,500 / 3,000 a year [D32].
5. **Price.** Patients treated × net price per patient gives AMT-130 revenue.
6. **Costs and cash items.** Cost of goods, operating expenses, capex, D&A, stock-based compensation (SBC), debt, cash, tax losses (NOLs) and share count are set by rule or by analyst instruction (Section 8, Figure 7).
7. **Valuation.** Finagle discounts free cash flow to equity (FCFE) at the cost of equity, with 2% terminal growth, to give a value per share.

How to read the source tags. Every number carries a tag in square brackets that points to Appendix B. **A** is a reported actual; **G** is a company statement or guidance; **E** is an external estimate (analysts, papers); **D** is a document or data file; **S** is an assumption we made; **C** is a calculation from tagged inputs; **P** is an analyst instruction that sets a modelling choice. Section 10 (confirmatory-study cost) adds **B** for a published benchmark, **I** for our own inference with the arithmetic shown, and **K** for the output of a CEO-simulation bot, which is a simulation and not a company statement. An entry marked "unverified" means the page or link was not reopened for this report. "Judgment" in the text marks the analyst's view rather than a sourced fact. In short: A, G, D and E are evidence; S and P are choices; C is arithmetic on the others.

The assumptions that matter most.

- **Approval on 1 July 2027 (base and bull)** – judgment. The FDA accepted the BLA with Priority Review and a PDUFA date of 1 May 2027 [G701]; launch two months later allows for the action date and launch preparation. Not probability-weighted [G12].
- **Confirmatory-study cost of $129M over FY2027E–FY2031E ($30M / $45M / $32M / $17M / $5M on a separate line), the same in all three scenarios** – our inference; uniQure has not disclosed the cost. Section 10 sets out the evidence (uniQure's own trial spending, Aspen Neuroscience's IPO prospectus, the AskBio and BlueRock trials, published benchmarks and a labelled CEO-simulation view) and lower and upper bands [I1][P110].
- **A launch pool of ~6,000 US patients** – evidence: a company management estimate of patients "addressable" at launch, not a treatment target [G5].
- **Leave rate m = 0.62 a year** (an average stay in the eligible window of ~1.6 years) – judgment. It was chosen because it makes disease incidence consistent with the 6,000. The clinical evidence (duration studies and the trial's crossover data) points nearer 0.41 a year, a stay of ~2.4 years [A23][C510].
- **72.9% of eligible patients treated** (90% referred × 90% willing × 90% covered) – assumption [S2].
- **Capacity of 330 / 2,500 / 3,000 a year** – a ClearPoint forecast, judgment informed by management comments, not a company figure [D32].
- **Net price of $2.0M per patient** – assumption, below comparable list prices of $3.2M–$4.25M [A13][S9].
- **Valuation rules** (2% terminal growth, 1x EBITDA debt, equity raises so cash never goes negative) – analyst instructions and judgment [P103][P105][P109].

Glossary

Term	Meaning in this report
AMT-130	uniQure's one-time AAV5 gene therapy for Huntington's disease
HD	Huntington's disease, an inherited neurodegenerative disease
TFC	Total Functional Capacity, a 0–13 score of daily functioning in HD; the trial enrolled TFC 9–13
Early-manifest	The first symptomatic stage of HD; the pivotal trial defined it by TFC 9–13 [D602]; the treatable window in this report
Pool	US patients eligible for AMT-130 and not yet treated, at a point in time
Entrants	Patients who join the pool in a period (newly eligible)
Treated	Patients who receive AMT-130 in a period
Capacity	The maximum number of procedures centres can perform in a period
Leave rate m	The share of the pool that leaves each year by progressing out of eligibility or dying
P	The share of entrants that is treated (assumed 0.729 when demand, not capacity, limits)
u	The treatment rate on the pool: treated divided by the pool
g	The pool's growth rate; g* = 0.5% a year is population growth
Consensus	The average of sell-side analyst forecasts
EBITDA	Earnings before interest, tax, depreciation and amortisation
D&A	Depreciation and amortisation
Capex	Capital expenditure
SBC	Stock-based compensation
NOL	Net operating loss carried forward, used to offset future taxable income
FCFE	Free cash flow to equity: cash available to shareholders after costs, tax, capex and debt flows
DDM	Dividend discount model; here it gives the same value as FCFE
WACC	Weighted average cost of capital; Finagle reports it alongside the cost of equity
Terminal growth	The growth rate assumed for cash flows after the last forecast year
CAGR	Compound annual growth rate
BLA / MAA	US Biologics License Application / UK Marketing Authorisation Application

1. Summary

The four points below give the forecast and valuation in brief; Figure 1 gives the base-case numbers.

FORECAST SUMMARY

■ **US AMT-130 volumes come from an eligible-pool model at a leave rate of 0.62 a year, starting from uniQure's ~6,000 launch pool, and are capped by a ClearPoint capacity ramp: 330 patients in FY2027E, 2,500 in FY2028E and 3,000 a year from FY2029E [P9][S5].** Treated patients stay at the 3,000 cap through FY2030E and ease to 2,840 by FY2032E, when demand from the pool, not capacity, becomes the limit [C98].

■ **Base revenue is $5,035M in FY2028E and about $6.0B in FY2029E–FY2030E [C21][C23],** and stays above $5B through FY2033E. Net price is $2.0M per patient, 10% lower from FY2031E [S9]. This is far above the Street: FY2027E consensus is about one-sixth of our build (consensus divided by build is 0.165) [C236], and Goldman's reported peak is $2.5B [E6].

■ **EBITDA turns positive in FY2027E at $280M and settles near $4.0B, a margin of about 77% [S23][C43].** Bull reaches $9.9B of revenue in FY2028E on 4,500 patients, then runs at a 3,500 cap. Bear assumes no approval: royalties continue and costs are cut.

■ **Finagle values the shares at $335.65 in base, $560.67 in bull and -$0.78 in bear, against a price of $25.30 on 2026-10-30 [D45].** The base value is about 13x the price [C509]. That gap reflects volumes far above consensus and an assumed approval; it should be read with the risks in Section 6 and the sensitivities in Section 9. The key open question is the leave rate: 0.62 a year is shorter-stay than the trial crossover data, which point to about 0.41 [A23].

Figure 1 – Financial summary, base case

Row (units)	FY25A	FY26E	FY27E	FY28E	FY29E	FY30E	FY31E	FY32E
US patients treated (patients/yr)	–	0 [C92]	330 [C93]	2,500 [C94]	3,000 [C95]	3,000 [C96]	2,984 [C97]	2,840 [C98]
Revenue (US$M)	16.1 [A7]	24.9 [C8]	690 [C20]	5,035 [C21]	6,040 [C22]	6,044 [C23]	5,419 [C24]	5,162 [C25]
Revenue growth (%, derived) [C401]	–	54.8%	2,669.3%	629.7%	20.0%	0.1%	-10.3%	-4.7%
Gross margin (%)	89.5% [C32]	93.9% [C33]	85.4% [C34]	86.1% [C35]	87.0% [C36]	88.0% [C37]	87.8% [C38]	88.9% [C39]
EBITDA (US$M)	-153 [C28]	-161 [C31]	280 [S23]	3,719 [S24]	4,594 [S25]	4,666 [S26]	4,142 [S27]	3,984 [S28]
EBITDA margin (%, derived) [C401]	-949.9%	-645.9%	40.6%	73.9%	76.1%	77.2%	76.4%	77.2%

Source: forecast file, base case [D46]; growth and margins derived [C401]. FY2026E revenue is consensus. **What this means:** revenue steps up by more than an order of magnitude in FY2027E–FY2028E as AMT-130 launches, and the EBITDA margin moves from losses to about three-quarters of revenue because a one-time therapy at $2.0M carries low per-patient costs.

2. Business Description and Reporting Lines

What uniQure sells today and how the reported revenue lines map to the forecast.

uniQure is a gene-therapy company. Almost all of its revenue today is royalties on Hemgenix, the haemophilia B gene therapy licensed to CSL Behring [A21]. AMT-130 for Huntington's disease (HD) is a one-time AAV5 gene therapy given by MRI-guided neurosurgery with ClearPoint Neuro's system; its BLA and a UK MAA were filed in September 2026 [G12]. The reported lines are license (royalty), collaboration and contract manufacturing revenue; we add AMT-130 for the US and, in bull only, the UK. Genezen makes AMT-130 and gives uniQure preferential access to its plant [G13].

Figure 2 – Revenue by line, FY2025A

Line	FY2025A (US$M)	Share of revenue	How uniQure earns
License (Hemgenix royalties)	15.9 [A2]	99%	royalty from CSL Behring
Collaboration	0.2 [A4]	1%	services to partners, wound down
Contract manufacturing	0.0 [A25]	0%	divested to Genezen in 2024 [G13]
Total	16.1 [A7]	100%	

Source: 10-K FY2025 [D1]; shares derived [C401].

3. Management

Who sets the launch plan and what management has and has not said about capacity.

CEO Matt Kapusta and Chief Customer & Strategy Officer Kylie O'Keefe set out the launch plan, including about 6,000 "initially treatable" US patients at launch [G5][G6]. Management gives no capacity number; it says capacity depends on neurosurgeons and operating suites [G9]. Its record on commercial launches is thin: Hemgenix was out-licensed, so AMT-130 would be uniQure's first own launch. We do not assess ownership or deal record in this update.

4. Key Developments

The dated events that set the timeline in the forecast.

Figure 3 – Key developments

Date	Event	Source
2025-09-24	Investor event: ~6,000 initially treatable US patients at launch (management estimate)	[G5]
2026-07-29	Q2 2026: confirmatory-study design agreed with FDA; cash runway into 2030	[G4][A22]
2026-08-10	ClearPoint: 15 centers qualified for 1–2 procedures a week	[G1]
2026-09-02	BLA submitted; UK MAA filed, priority review requested	[G12]
2026-10-28	ClearPoint: approval possible mid to second half of 2027	[G3]
2026-11-02	FDA accepts the BLA (Accelerated Approval, Priority Review), PDUFA date 1 May 2027, advisory committee tentatively planned; confirmatory study of ~200 patients, US screening by end-2026	[G701][E701]
2026-11-04	Q3 2026 results due	[S402]

Source: company filings and calls; ClearPoint statements.

What matters most is the size and turnover of the treatable pool. uniQure's 6,000 is an estimate of patients addressable at launch, not a target to treat, and the company has not said how quickly patients leave the treatable window [G5][G6].

5. Business Drivers by Line

The assumptions behind each revenue and cost line. AMT-130 volume is the driver that matters; the others are small or mechanical.

Why a pool model. AMT-130 is given once, and only to patients in the early stage of HD. A patient who is treated, progresses past the eligible stage or dies does not come back. So the number who can be treated in any year depends on how many eligible patients are waiting, how many newly become eligible, and how fast untreated patients age out. A pool model tracks exactly that. **Why capacity limits the early years.** Each treatment is an MRI-guided neurosurgical procedure at a qualified centre. At launch there are far more eligible patients waiting (~6,000) than centres can treat, so through FY2030E the number treated is set by capacity; only later, once the waiting pool is worked down, does patient demand set the number. **Why m = 0.62 is a judgment call.** At 0.62 a year, incidence alone produces a pool of about 6,000, matching uniQure's estimate with no extra assumption. But the clinical evidence points to patients staying eligible longer, nearer 0.41 a year (Appendix D.6). A slower leave rate would mean a larger pool than 6,000 unless some other eligibility filter applies, and the choice changes long-run demand.

AMT-130 US: volume. Treated patients each quarter are the lower of capacity and pool demand [S5]. The pool starts at 6,000 on 1 July 2027 [G5]. Each year about 3,788 new patients enter it from HD incidence, net of a referral share [C2]. Untreated patients leave at 0.62 a year (progression 0.60 plus death 0.02), an average stay of about 1.6 years [P9]. Of eligible patients, 72.9% are treated once demand, not capacity, sets volume: 90% referred, 90% willing and 90% covered, all our assumptions [S2]. Capacity follows the ClearPoint forecast ramp of 330 / 2,500 / 3,000 a year [D32][S5].

Price and recognition. Base net price is $2.0M through FY2030E, then 10% lower once US sales pass $2B a year [S9]. Bull is $2.4M, stepping down from FY2028E [S10]. List prices for comparable gene therapies run $3.2M–$4.25M [A13]. We recognise revenue at treatment [S401].

Costs. COGS per patient is $0.30M at launch, falling to $0.20M; it is unverified [S11]. Opex is the Q2 2026 run rate [A22] plus US commercial fixed cost, $1.5M per center, a 5% variable cost and a separate confirmatory-study line of $30M, $45M, $32M, $17M and $5M in FY2027E–FY2031E [I1]; how that cost was estimated, and its lower and upper bands, are set out in Section 10. The same line is carried in bull and bear [P110]. Centers for that cost line are derived from capacity at two procedures a week (about 29 in base), not from a company count [S5].

Hemgenix royalties. These grow to $52.2M in FY2033E [S19]. The cash goes to HemB SPV under the royalty financing [A14]; by analyst instruction, the $473.2M royalty-financing liability is not treated as debt in the valuation [P105].

6. Risks

The main ways the forecast could be wrong, in rough order of importance. The valuation is about 11x the share price, so each of these matters.

Approval. Base and bull assume US approval and launch on 1 July 2027; bear assumes no approval. The valuation is not probability-weighted, so the base value is conditional on approval [G12]. The FDA accepted the BLA on 2 November 2026 with Priority Review and a PDUFA target action date of 1 May 2027 [G701]. Acceptance is not approval: the FDA tentatively plans an advisory committee, and the outcome is open. Accelerated approval would also depend on the confirmatory study, which bear still carries in cost though bear assumes no approval.

Pool turnover. The leave rate of 0.62 is the main lever. At about 0.40 a year (a 2.5-year stay), the 6,000 would need an unexplained eligibility filter, and base treated patients would fall to about 1,850 a year in the long run (scratch run, not in the forecast file) [P9]. If new entrants are only 30% of incidence, base treated falls to 927 in FY2030E and revenue to $1,899M (Figure 4).

Capacity. The 330 / 2,500 / 3,000 ramp is our ClearPoint forecast, not a company figure [D32]. Capacity binds through FY2030E in base and in every year in bull.

Approval timing. If approval slips to Q4 2027, base FY2027E revenue is $360M (Figure 4).

Payer budget. US spend of about $6B a year from FY2029E invites managed access.

Consensus. FY2027E consensus is about one-sixth of our line build (consensus divided by build is 0.165), well outside the 10% band in which we would scale; we do not scale it [C236]. Goldman's reported peak is $2.5B [E6].

Figure 4 – Sensitivities (US patients; US$M)

Each block shows one case against the filed forecast: approval slipping to Q4 2027, and new entrants at only 30% of incidence.

Case (units)	FY27E	FY28E	FY29E	FY30E	FY31E	FY32E	FY33E
base filed: US treated (patients/yr)	330	2,500	3,000	3,000	2,984	2,840	2,832
base filed: revenue (US$M)	690	5,035	6,040	6,044	5,419	5,162	5,149
base filed: EBITDA (US$M)	280	3,719	4,594	4,666	4,142	3,984	3,967
base approval Q4 2027 (2027-10-01): US treated (patients/yr)	165	2,040	2,875	3,000	3,000	2,875	2,833
base approval Q4 2027 (2027-10-01): revenue (US$M)	360	4,115	5,790	6,044	5,448	5,226	5,151
base approval Q4 2027 (2027-10-01): EBITDA (US$M)	15.8	2,974	4,389	4,666	4,165	4,037	3,968
base entrants = 30% of incidence: US treated (patients/yr)	330	2,500	1,466	927	910	909	909
base entrants = 30% of incidence: revenue (US$M)	690	5,035	2,973	1,899	1,685	1,686	1,689
base entrants = 30% of incidence: EBITDA (US$M)	280	3,719	2,078	1,225	1,050	1,068	1,064
bull filed: US treated (patients/yr)	880	4,500	3,500	3,500	3,500	3,500	3,500
bull filed: revenue (US$M)	2,142	9,930	8,594	8,774	8,778	8,397	8,016
bull filed: EBITDA (US$M)	1,473	7,746	6,727	6,984	7,100	6,868	6,546
bull approval Q4 2027 (2027-10-01): US treated (patients/yr)	440	3,815	3,750	3,500	3,500	3,500	3,500
bull approval Q4 2027 (2027-10-01): revenue (US$M)	1,086	9,366	9,134	8,774	8,778	8,397	8,016
bull approval Q4 2027 (2027-10-01): EBITDA (US$M)	602	7,399	7,178	6,984	7,100	6,868	6,546
bull entrants = 30% of incidence: US treated (patients/yr)	880	4,037	1,365	1,392	1,392	1,392	1,392
bull entrants = 30% of incidence: revenue (US$M)	2,142	8,931	3,983	4,221	4,224	3,844	3,463
bull entrants = 30% of incidence: EBITDA (US$M)	1,473	6,924	2,880	3,133	3,196	2,922	2,599

Source: forecast file sensitivities [D46]. **What this means:** an approval delay mainly moves FY2027E–FY2028E, while lower entrants cut long-run volume and revenue, which is what carries the valuation.

7. Forecast

How the evidence becomes the forecast numbers, and the base case line by line.

How the evidence becomes the numbers. The forecast is a quarterly line build. The US patient demand model is set out in full in Appendix D. FY2026E is consensus because there is no guidance. From FY2027E, US AMT-130 revenue is patients treated times net price; patients come from the pool model and the capacity ramp. Totals are the line build, not consensus.

Figure 5 – Evidence → assumption → forecast impact

Read across each row: the evidence, the assumption we drew from it, and what that does to the forecast.

Evidence	Assumption	Forecast impact
~6,000 initially treatable US patients at launch, management estimate [G5][G6]	Start pool 6,000 on 2027-07-01	Launch stock worked down to ~2,000 by FY2030E
Analyst choice of m = 0.62 as the most internally consistent leave rate [P9]	Leave rate 0.62/yr (stay ~1.6 yr)	Steady pool ~6,000 with no extra filter; long-run treated ~2,840/yr
Crossover: 2 of 6 high-dose controls failed inclusion after ~12 months [A23]	Not used; points to ~0.41/yr	Flagged as the main downside to demand
HD incidence [S4]; referral/willing/covered 90% each [S2]	Entrants ~3,800/yr; treated share 72.9%	Sets long-run demand
ClearPoint capacity ramp [D32]	330 / 2,500 / 3,000 cap	Binds FY2027E–FY2030E
Comparable list prices [A13]	$2.0M net, −10% from FY2031E [S9]	Revenue ~$6.0B FY2029E–FY2030E
Q2 2026 opex run rate [A22]; confirmatory study [I1]	Opex build	EBITDA margin ~75–77%
FY2027E consensus [C236]	Not scaled	Build far above consensus

Source: as tagged.

Base case. Figure 6 shows the base case by line: reported years, consensus FY2026E, then the line build. Totals are the sum of the lines.

Figure 6 – Base case by line, FY2024A–FY2033E

Row (units)	FY24A	FY25A	FY26E	FY27E	FY28E	FY29E	FY30E	FY31E	FY32E	FY33E
US centers implied by capacity at 2/week (count)	–	–	0 [C44]	6 [C45]	24 [C46]	29 [C47]	29 [C48]	29 [C49]	29 [C50]	29 [C51]
US treatment capacity (patients/yr)	–	–	0 [C52]	330 [C53]	2,500 [C54]	3,000 [C55]	3,000 [C56]	3,000 [C57]	3,000 [C58]	3,000 [C59]
US entrants to eligible pool (patients/yr)	–	–	0 [C60]	1,884 [C61]	3,788 [C62]	3,806 [C63]	3,826 [C64]	3,845 [C65]	3,864 [C66]	3,883 [C67]
US pool demand (patients/yr)	–	–	0 [C68]	4,980 [C69]	8,332 [C70]	5,686 [C71]	3,982 [C72]	3,138 [C73]	2,840 [C74]	2,832 [C75]
US untreated pool leavers (patients/yr)	–	–	0 [C76]	1,836 [C77]	3,072 [C78]	2,097 [C79]	1,468 [C80]	1,157 [C81]	1,047 [C82]	1,044 [C83]
US eligible pool at year end (patients)	–	–	6,000 [C84]	5,718 [C85]	3,933 [C86]	2,643 [C87]	2,000 [C88]	1,703 [C89]	1,680 [C90]	1,687 [C91]
US patients treated (patients/yr)	–	–	0 [C92]	330 [C93]	2,500 [C94]	3,000 [C95]	3,000 [C96]	2,984 [C97]	2,840 [C98]	2,832 [C99]
US cumulative treated (patients)	–	–	0 [C100]	330 [C101]	2,830 [C102]	5,830 [C103]	8,830 [C104]	11,814 [C105]	14,654 [C106]	17,486 [C107]
License revenue, Hemgenix royalties (US$M)	10.1 [A1]	15.9 [A2]	24.9 [C8]	30.0 [S13]	35.0 [S14]	40.0 [S15]	44.0 [S16]	47.5 [S17]	50.0 [S18]	52.2 [S19]
Collaboration revenue (US$M)	10.9 [A3]	0.2 [A4]	0.0 [S20]	0.0 [S20]	0.0 [S20]	0.0 [S20]	0.0 [S20]	0.0 [S20]	0.0 [S20]	0.0 [S20]
Contract manufacturing revenue (US$M)	6.1 [A5]	0.0 [A25]	0.0 [G14]	0.0 [G14]	0.0 [G14]	0.0 [G14]	0.0 [G14]	0.0 [G14]	0.0 [G14]	0.0 [G14]
AMT-130 revenue, US (US$M)	0.0 [A26]	0.0 [A26]	0.0 [S21]	660 [C11]	5,000 [C12]	6,000 [C13]	6,000 [C13]	5,372 [C14]	5,112 [C15]	5,097 [C16]
AMT-130 revenue, UK (US$M)	0.0 [A26]	0.0 [A26]	0.0 [S22]	0.0 [S22]	0.0 [S22]	0.0 [S22]	0.0 [S22]	0.0 [S22]	0.0 [S22]	0.0 [S22]
Total revenue (US$M)	27.1 [A6]	16.1 [A7]	24.9 [C8]	690 [C20]	5,035 [C21]	6,040 [C22]	6,044 [C23]	5,419 [C24]	5,162 [C25]	5,149 [C26]
Gross margin (%)	–	89.5% [C32]	93.9% [C33]	85.4% [C34]	86.1% [C35]	87.0% [C36]	88.0% [C37]	87.8% [C38]	88.9% [C39]	88.9% [C40]
EBITDA (US$M)	-149 [C27]	-153 [C28]	-161 [C31]	280 [S23]	3,719 [S24]	4,594 [S25]	4,666 [S26]	4,142 [S27]	3,984 [S28]	3,967 [S29]
EBITDA margin (%, derived from the two rows above)	-550.9%	-949.9%	-645.9%	40.6%	73.9%	76.1%	77.2%	76.4%	77.2%	77.0%

Source: forecast file [D46]. me 77.0% [C43], mc 83.0% [S30], gsnext 0.03% [S31]. May not sum due to rounding.

8. Valuation – Finagle

This section turns the forecast into a value per share. Finagle is an external valuation model: it takes the revenue and EBITDA paths from Sections 1–7, the balance-sheet starting point and a set of rates and rules, and discounts free cash flow to equity (FCFE). All valuation numbers below are copied from Finagle's result file; none are recomputed here.

Finagle values QURE at $335.65 a share in base, $560.67 in bull and -$0.78 in bear, vs. a price of $25.30 on 2026-10-30 [D45]. These are raw FCFE values; DDM gives the same figures, and no post-run adjustment was requested, so there is no adjusted value. No scenario is flagged not a valuation. We draw no rating or price target.

- **Validation warnings (Finagle's words):** base: "average EBITDA growth is 121.2% and there are no acquisitions. If that growth includes acquired EBITDA, use an organic path and book the purchase price with acquisitions, or MnA stays zero." [D45]. Base EBITDA growth is high because EBITDA turns from losses to about $4.6B over three years; there are no acquisitions in the forecast.
- **Sanity flags (Finagle's words):** bear: "negative FCFE after year 1" [D45]. Base and bull have none. Finagle's log is empty in all three scenarios.
- **Analyst-instructed inputs:** rd 10% [P101] (2026-10-29); debt at 1x EBITDA once EBITDA is positive, held in bear [P103] (2026-10-29); NOL 539 and the SBC rule [P104] (2026-10-30); year-0 D&A 0.2195, HemB liability not debt, terminal growth 2%, Dutch tax rate 25.8%, and cash and shares raised so cash never goes negative [P105] (2026-10-30); base and bull capex [0.439, 3.5, 3.5] then method 1 [P106] (2026-10-30); bear capex equal to base [P107] (2026-10-30); bear raise sized so cash stays positive [P109] (2026-10-30); year-0 cash = FY2025 cash plus short-term investments plus 2026 equity raises, with shares increased for those raises [P111] (2026-11-02); leave rate m = 0.62 and a valuation run on this forecast [P9][P8] (2026-10-30).
- **Defaults filled (Finagle's words):** baseline.noa, distribution.mode, forecast.dwc.mode, rates.re, rates.roict, year [D45]. Finagle also prints "forecast.sales_growth NOT IN CATALOG"; the revenue path still applies.
- **Fetched market inputs:** price $25.30 and risk-free rate 4.99%, both as of 2026-10-30; 10-K cover shares 62.5M [D45]. Year-0 cash and shares were set by us, not fetched (Figure 11).
- **Finagle commit** 377f177b, run 2026-11-02 11:40 (Toronto time). All sidecar hashes match (file bytes, whole request, base/bull/bear blocks, instruction inputs, forecast.json bytes) [D45].

Method. Finagle discounts free cash flow to equity at the cost of equity (10.49%, Finagle's default). It takes our revenue and EBITDA for FY2026E–FY2033E as given (eight explicit years after FY2025A) and extends them two more years to FY2035E using the starting margin, the incremental margin and next-period growth, fading to terminal growth of 2.00%, then adds a terminal value. Terminal growth of 2% is an analyst instruction, not Finagle's default [P105]. Negative cash flow is funded from the opening cash balance: year-0 cash is the 10-K cash and current investments plus the June 2026 offering, and we add an extra year-0 equity raise only where it is needed to keep cash from going negative, and Finagle values the raise through the extra cash and shares [D45].

Figure 7 – Valuation inputs

Each row is one input Finagle used, its value in each scenario, and where it came from. "Finagle default" means the model filled it; "analyst instruction" means it was set deliberately; "judgment" means we chose it without a source.

Input	Base	Bull	Bear	Type and source
Cost of equity	10.49%	10.49%	10.49%	Finagle default (rf 4.99%, 2026-10-30) [D45]
Cost of debt	10.00%	same	same	analyst instruction, 2026-10-29 [P101]
Tax rate	25.80%	same	same	analyst instruction, Dutch statutory rate, 2026-10-30 [P105]
Terminal growth	2.00%	same	same	analyst instruction, 2026-10-30 [P105] (Finagle's own default would be 4.93%)
Terminal ROIC	15.00%	same	same	Finagle default [D45]
Shares (M)	68.2	68.2	236.2	10-K 62.5 + June 2026 offering 5.7 + any extra raise at price [D48][D50][P111][S403]
Price (2026-10-30)	$25.30	same	same	Finagle fetch [D45]
Year-0 debt	49.7	same	same	10-K; HemB liability not debt [P105]
Year-0 cash	865	865	5,115	10-K cash + current investments + June 2026 offering = 865, + any extra raise [D48][D50][P111][P109]
Year-0 NOL	539	same	same	analyst instruction [P104]
Starting margin (me)	77.0%	81.7%	-174.0%	forecast file [D46]
Incremental margin (mc)	83.0%	86.6%	93.9%	forecast file [D46]
Next-period growth (gsnext)	0.03%	0.02%	3.00%	forecast file [D46]
Terminal WACC	10.08%	10.08%	10.51%	Finagle output [D45]
Capex	[0.439, 3.5, 3.5], then Finagle method 1	same	base dollars, all 11 years	analyst instruction [P106][P107]
Debt policy	1.0x EBITDA from FY2027E	same	hold	analyst instruction [P103]; start year follows the forecast
SBC	flat 17.7 until EBITDA is positive, then 5% of sales	same	flat 17.7	analyst rule [P104]; 5% is our judgment [S403]

Source: Finagle result [D45]; valuation request [D46]; analyst instructions [P101]–[P111]. **Judgment inputs.** Base and bull need no extra raise: with 865 of opening cash their lowest year-end cash is 678, so the extra raise is zero. The bear raise of 4,250 is the smallest $50M step that keeps bear cash from going negative with base-sized capex and the confirmatory-study cost; it exists only for that reason [P109][S403]. Employee option and ESPP proceeds of $6.985M in H1 2026 and the ~1.15M shares issued under employee plans are left out, because they are not equity raises [D50][S403]. SBC at 5% of sales applies the analyst's rule with a percentage we chose [S403].

Figure 8 – Value bridge

How firm value becomes value per share in each scenario. Read down a column: firm value, less debt plus cash, gives equity value, divided by shares.

Item (US$M unless stated)	Base	Bull	Bear
PV of explicit-period cash flows	not returned	not returned	not returned
Terminal value and share of total	not returned	not returned	not returned
Terminal-year FCFE	2,197	3,644	-556
Firm value	22,081	37,432	-5,249
Equity value	22,031	37,382	-5,299
Year-0 debt / cash	49.7 / 865	49.7 / 865	49.7 / 5,115
Shares (M)	68.2	68.2	236.2
Value per share (US$)	$335.65	$560.67	-$0.78
vs. price $25.30 [C509]	+1,227%	+2,116%	-103%

Source: Finagle result [D45]; upside derived [C509]. The result file does not split value into explicit-period and terminal PV, so those rows are left blank rather than recomputed. **What this means:** the base and bull values rest on the cash flows from FY2028E onward, when AMT-130 volumes are at or near capacity, so they depend on approval and on the volume path holding (judgment).

Figure 9a – Finagle valuation table, base case. *The year-by-year build behind the base value: revenue and EBITDA from the forecast, then the items Finagle derives (D&A, interest, tax, debt, cash) down to FCFE. Rows marked * come from Finagle's notebook replay in the result file.*

Ticker QURE; price $25.30; shares 68.2M; cost of equity 10.49%; cost of debt 10.00%; tax 25.80%; terminal growth 2.00%.

Row (US$M unless stated)	FY2025A	FY2026E	FY2027E	FY2028E	FY2029E	FY2030E	FY2031E	FY2032E	FY2033E	FY2034E	FY2035E
Revenue	16.1	24.9	690	5,035	6,040	6,044	5,419	5,162	5,149	5,151	5,203
EBITDA	-153	-161	280	3,719	4,594	4,666	4,142	3,984	3,967	3,968	4,011
SBC*	17.7	17.7	34.5	252	302	302	271	258	257	258	260
D&A (derived)*	0.2	0.3	9.4	68.7	82.4	82.4	73.9	70.4	70.2	70.2	70.9
Capex	0.4	3.5	3.5	419	515	515	456	431	430	430	435
Interest*	62.0	5.0	5.0	28.0	372	459	467	414	398	397	397
Pre-tax income*	-233	-184	231	3,371	3,837	3,822	3,330	3,241	3,241	3,243	3,283
NOL balance*	539	723	492	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0

Source: Finagle result [D45].

Figure 9b – Finagle valuation table, base case (continued)

Row (US$M unless stated)	FY2025A	FY2026E	FY2027E	FY2028E	FY2029E	FY2030E	FY2031E	FY2032E	FY2033E	FY2034E	FY2035E
NOL used*	0.0	-184	231	492	0.0	0.0	0.0	0.0	0.0	0.0	0.0
Tax expense*	5.6	-0.0	59.6	870	990	986	859	836	836	837	847
Cash taxes*	5.6	0.0	-0.0	743	990	986	859	836	836	837	847
Debt	49.7	49.7	280	3,719	4,594	4,666	4,142	3,984	3,967	3,968	4,011
Cash	865	678	1,145	6,862	10,151	12,627	14,191	16,078	18,105	20,153	22,269
FCFE	-239	-187	467	5,717	3,289	2,475	1,565	1,887	2,028	2,048	2,116
WACC*	10.48%	10.48%	10.46%	10.07%	9.98%	9.97%	10.03%	10.06%	10.07%	10.07%	10.08%
Value per share (US$)*	$335.65	$359.58	$390.45	$347.60	$335.85	$334.78	$346.96	$355.70	$363.29	$371.37	$379.31

Source: Finagle result [D45]. M&A and working-capital change are zero in every year (no acquisitions; Finagle's default working-capital mode). Debt is exactly 1.0x EBITDA from FY2027E.

Figure 9c – Free cash flow, base case, FY2026E–FY2029E

Row (US$M)	FY2026E	FY2027E	FY2028E	FY2029E
EBITDA	-160.9	279.8	3,719.1	4,593.5
of which confirmatory study [I1]	0.0	-30.0	-45.0	-32.0
Capex	-3.5	-3.5	-419.0	-515.1
SBC, counted as cash*	-17.7	-34.5	-251.7	-302.0
Interest*	-5.0	-5.0	-28.0	-371.9
Cash tax*	0.0	0.0	-742.7	-990.0
FCF	-187.1	+236.9	+2,277.7	+2,414.5
Debt drawn (FCFE - FCF) [C703]	+0.0	+230.1	+3,439.3	+874.4
FCFE	-187.1	+467.0	+5,717.0	+3,288.9
Year-end cash	678.2	1,145.2	6,862.2	10,151.1

*Source: Finagle result [D45]; debt drawn and the tax check derived [C703]. FCF is EBITDA less capex, SBC (treated as a cash cost), interest and cash tax; FCFE adds the debt drawn under the 1x EBITDA rule. Rows marked * come from Finagle's notebook replay.* **What this means:** the only cash-burning year in base is FY2026E, which is pre-launch. FY2028E is where capex steps up and the tax-loss carryforward is used up. The confirmatory study (Section 10) is already inside EBITDA: $30M in FY2027E, $45M in FY2028E and $32M in FY2029E. In FY2027E the tax-loss carryforward shelters profit, so the cost lowers FCF dollar for dollar; from FY2028E, when cash tax is paid, part of it comes back as lower tax.

Figure 10a – Key rows, bull

The same build for bull and bear, limited to the rows that move value most.

Row (US$M unless stated)	FY2025A	FY2026E	FY2027E	FY2028E	FY2029E	FY2030E	FY2031E	FY2032E	FY2033E	FY2034E	FY2035E
Revenue	16.1	24.9	2,142	9,930	8,594	8,774	8,777	8,397	8,016	8,017	8,098
EBITDA	-153	-161	1,473	7,746	6,727	6,984	7,100	6,868	6,546	6,547	6,617
D&A (derived)*	0.2	0.3	29.2	135	117	120	120	114	109	109	110
Capex	0.4	3.5	3.5	933	773	795	795	750	704	704	714
NOL balance*	539	723	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
Cash taxes*	5.6	0.0	157	1,797	1,395	1,484	1,508	1,451	1,380	1,389	1,405
Debt	49.7	49.7	1,473	7,746	6,727	6,984	7,100	6,868	6,546	6,547	6,617
Cash	865	678	3,303	13,948	16,284	20,135	23,911	27,217	30,268	33,667	37,175
FCFE	-239	-187	2,624	10,645	2,336	3,851	3,776	3,306	3,051	3,400	3,508

Source: Finagle result [D45].

Figure 10b – Key rows, bear

Row (US$M unless stated)	FY2025A	FY2026E	FY2027E	FY2028E	FY2029E	FY2030E	FY2031E	FY2032E	FY2033E	FY2034E	FY2035E
Revenue	16.1	24.9	30.0	35.0	40.0	44.0	47.5	50.0	52.2	53.8	55.2
EBITDA	-153	-161	-172	-162	-134	-116	-100	-93.0	-90.9	-89.4	-88.2
D&A (derived)*	0.2	112	176	239	303	353	397	429	458	477	494
Capex	0.4	3.5	3.5	419	515	515	456	431	430	430	435
NOL balance*	539	835	1,205	1,629	2,089	2,580	3,101	3,646	4,217	4,806	5,412
Cash taxes*	5.6	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
Debt	49.7	49.7	49.7	49.7	49.7	49.7	49.7	49.7	49.7	49.7	49.7
Cash	5,115	4,928	4,730	4,126	3,454	2,801	2,222	1,675	1,132	589	43.6
FCFE	-239	-187	-198	-604	-672	-654	-579	-547	-543	-542	-546

Source: Finagle result [D45].

Cash, debt and tax. Cash never goes negative. Minimum year-end cash is base 678 (FY2026E), bull 678 (FY2026E) and bear 43.6, reached at the end in FY2035E [D45]. No payout is modelled, so base cash builds to 22,269 and bull to 37,175 by FY2035E. Base and bull debt sit at exactly 1.0x EBITDA from FY2027E. Base debt peaks at 4,666 and bull at 7,746; bear is held at 49.7 [D45]. The opening NOL offsets 100% of pre-tax income with no Dutch cap, which is generous: base pays no cash tax through FY2027E, bull uses its NOL in FY2027E, and bear's NOL grows to 5,412 [D45].

Figure 11 – Year-0 cash and shares: reconciliation and extra raise

Finagle funds negative cash flow from opening cash. Year-0 cash starts from the 31 December 2025 balance sheet (cash plus current investment securities) and adds the June 2026 offering; shares add the offering shares. A dry run with that opening cash shows the lowest year-end cash; where it is negative, an extra raise is added at the current price in $50M steps until cash never goes negative. The extra raise has no other purpose.

US$M unless stated	Base	Bull	Bear	Source
Cash and cash equivalents, 31 Dec 2025	80.240	same	same	10-K balance sheet [D48]
Current investment securities, 31 Dec 2025	542.301	same	same	10-K balance sheet [D48]
June 2026 follow-on offering, net proceeds	242.681	same	same	10-Q [D50]; 8-K [D49]
Year-0 cash before any extra raise	865.222	same	same	sum [C511]
Lowest year-end cash with no extra raise (dry run)	678.15	678.15	-4,206.41	scratch Finagle compile [D46]
Extra year-0 raise at US$25.30	0	0	4,250	rule [P109][S403]
Year-0 cash used	865.222	865.222	5,115.222	valuation request [D45]
Lowest year-end cash in the valuation	678.15	678.15	43.59	Finagle result [D45]
Shares on 10-K cover (M)	62.529408	same	same	10-K cover [D45]
June 2026 offering shares (M)	5.686813	same	same	10-Q [D50]
Extra-raise shares (M)	0.000000	0.000000	167.984190	raise / 25.30 [C504]
Shares used (M)	68.216221	68.216221	236.200411	valuation request [D45]

Source: Finagle result [D45]. Extra-raise shares issued at the fetched price with no discount or fees [S403]. The bear raise is about 2.5x the market cap of 1,726 on 68.2M shares [C508]. Hercules debt stays at 49.7 with no new borrowing; the HemB royalty financing liability (about $479M) is treated as not debt, per the analyst instruction [P105].

Figure 12 – Scenarios vs. price

Headline valuation outputs and balance-sheet checks for the three scenarios side by side.

Item (US$M unless stated)	Base	Bull	Bear
Value per share, FCFE (US$)	$335.65	$560.67	-$0.78
Value per share, DDM (US$)	$335.65	$560.67	-$0.78
Equity value	22,031	37,382	-5,299
Firm value	22,081	37,432	-5,249
Terminal-year FCFE	2,197	3,644	-556
Terminal WACC	10.08%	10.08%	10.51%
Shares (M)	68.22	68.22	236.20
Year-0 cash	865	865	5,115
Minimum cash	678	678	43.59
Debt peak	4,666	7,746	49.70
Max debt / EBITDA (x)	1.0x	1.0x	n/a
Flagged not a valuation	no	no	no
Value per share vs. price $25.30 [C509]	+1,227%	+2,116%	-103%

Source: Finagle result [D45]; upside derived [C509]. The spread between scenarios is driven by approval (bear assumes none) and, between base and bull, by price and capacity.

Caveats.

- **Bear.** EBITDA is negative in every year, so FCFE is negative after year 1 and equity value is -5,299. The -$0.78 rests on perpetual losses and on the assumed 4,250 extra raise. Bear D&A exceeds capex from FY2033E and reaches 494 vs capex 435 and revenue 55.2 in FY2035E, because base-sized capex sits on bear's small revenue. It is positive, so Finagle does not flag it, but it inflates bear's NOL build-up. Read bear as roughly zero.
- **Funding.** Base and bull are funded by existing cash, investments and the June 2026 offering, with no extra raise; the bear raise implies heavy dilution (168M new shares on 68.2M). Fees, discount and timing are not modelled. Employee-plan cash and shares and any pre-funded warrants are left out [S403].
- **Cash accumulation.** With no distribution, cash piles up from FY2028E; Finagle values it through FCFE, but the report makes no assumption about its use.
- **Terminal growth vs. gsnext.** Terminal growth is set at 2% by analyst instruction [P105]; the forecast's next-period growth is 0.03% base, 0.02% bull and 3.00% bear. The two have not been reconciled.
- **Approval** is not probability-weighted; base and bull assume approval on 1 July 2027.
- **Demand.** The valuation inherits the pool model's m = 0.62, which is shorter-stay than the trial crossover evidence (Section 9 and Appendix D).

9. What Drives the Valuation: Key Assumptions and Sensitivities

This section pulls together the assumptions that move the value most, what evidence supports each, and what happens if it is wrong. All numbers are from earlier sections and Appendix D.

Figure 13 – Key assumptions, evidence and sensitivity

Assumption	Value used	Evidence or judgment	Effect if wrong
Approval and launch	1 July 2027 (base, bull)	Judgment from filing and review timing [G12]	Q4 2027 approval: base FY2027E revenue $360M (Figure 4); no approval is the bear case at -$0.78 [D45]
Launch pool	~6,000	Company management estimate [G5]	Criteria behind it undisclosed
Leave rate m	0.62/yr (stay ~1.6 yr)	Judgment; evidence points nearer 0.41 [A23][C510]	At ~0.40 with a filter on all entrants, long-run treated ~1,850/yr [P9]
New entrants	~3,800/yr	Incidence [S4] and 90% early-manifest share (assumption)	At 30% of incidence, base FY2030E treated 927 (Figure 4)
Treated share P	0.729	Assumption [S2]	Scales long-run volume one-for-one
Capacity	330 / 2,500 / 3,000	ClearPoint forecast, judgment [D32]	Binds through FY2030E; bull's faster ramp front-loads volume
Net price	$2.0M, −10% from FY2031E	Assumption below comparable list prices [A13][S9]	Revenue moves one-for-one
Terminal growth	2%	Analyst instruction [P105]	Not reconciled with the forecast's next-period growth
Confirmatory-study cost	Separate line $129M over FY2027E–FY2031E, all scenarios	Inference from benchmarks; cost undisclosed [I1]	Bands $63M–$222M: base value per share $336.18 to $334.90; bear raise $4,150M to $4,300M [D45] [D47] (Figure 17)
Extra year-0 raises	$0 / $0 / $4,250M	Rule: only to keep cash from going negative [P109] [S403]	Bear raise is about 2.5x market cap [C508]

Source: as tagged. **What this means:** the base value is conditional on approval, and after approval the leave rate and treated share decide how many patients are treated once capacity stops binding. Those two are assumptions, not company figures.

Demand-method caveats.

- **m = 0.62 is a choice.** It was picked as the most internally consistent option, because at that leave rate incidence alone gives a steady pool of about 6,000 [P9]. It implies a 1.6-year stay, shorter than the duration evidence. In the trial, 2 of the 6 high-dose controls no longer met inclusion criteria after about 12 months [A23], which implies a leave rate of about 0.41 a year (derived; six patients, so the range is wide) [C510].

- **6,000 is not a treatment target.** uniQure calls it "Initial Treatable Patients at Launch", a management estimate of patients addressable at launch, about 30% of ~20,000 diagnosed patients [G5][G6]. The criteria behind the 30% are not disclosed.
- **0.729 is an assumption.** The treated share is 90% referred × 90% willing × 90% covered; none is a company figure [S2].

Valuation mechanics to note.

- **Debt** is set at 1x EBITDA from FY2027E, the first year EBITDA is positive [P103][D46].
- **SBC** is 5% of sales once EBITDA is positive [P104][S403].
- **Bear capex** equals base capex in dollars, which is large against bear's small revenue [P107].
- **Bear raise** is $4,250M, about 2.5x the ~$1.73B market cap, to keep bear cash positive [P109][C508].

Open flags.

- FY2027E is not scaled to consensus (consensus / build 0.165, outside our 10% rule) [C236].
- Year-0 cash excludes $6.985M of employee option and ESPP proceeds and the related ~1.15M shares [D50][S403].
- Cash accumulates with no distribution modelled [D45].
- Terminal growth of 2% is not reconciled with the forecast's next-period growth (0.03% base) [P105][D46].
- Centers are derived from capacity at two procedures a week, only for the opex line [S5].

10. Confirmatory Study Cost: Analysis and Cost Bands

Purpose and scope

uniQure has asked the FDA for accelerated approval of AMT-130 (ifezuntirgene inilparvovec) in Huntington's disease. Accelerated approval is granted on early evidence and must be followed by a confirmatory study that verifies clinical benefit. On 2 November 2026 the company said this study will enrol about 200 patients [G701]. They will be randomised two-to-one between AMT-130 and standard of care, which means usual medical management with no surgery. The primary endpoint is Total Functional Capacity (TFC, a standard rating of everyday function in Huntington's disease) at 36 months, and US screening is to begin by the end of 2026. uniQure has not disclosed a budget for the study. This section estimates what the study will cost, sets a lower and an upper band around a central estimate, and states the exact amount to carry on the forecast's separate confirmatory-study line. The same profile applies in the bear, base and bull cases. The bear case assumes the drug is not approved, but the study is assumed to be committed and paid for regardless, so the cost does not change between cases.

Every figure carries a tag. This section uses the report's D, S and G tags plus three of its own:

- **D** is disclosed in a filing, release or official registry, and **G** is a uniQure company statement.
- **B** is a published benchmark.
- **I** is our own inference, with the arithmetic shown.
- **S** is an assumption.
- **K** is the output of a CEO-simulation bot (the Matt Kapusta bot, a simulation of uniQure's chief executive). The bot reasons from public filings in the manner of uniQure's chief executive. It is not a company statement.

In the rest of the report C marks our arithmetic; in this section that arithmetic is tagged I so that each inference can be traced step by step. Sources are numbered and listed in Appendix B with the report's other sources.

What the study involves

About 133 patients will receive AMT-130 and about 67 will act as controls. A treated patient undergoes MRI-guided neurosurgery. During the operation, catheters deliver a one-time dose of the AAV vector (adeno-associated virus, the harmless virus used to carry the gene) into the striatum, the brain region Huntington's disease damages first. The surgery uses ClearPoint Neuro's navigation system and SmartFlow cannula, and it needs MRI-suite time, a neurosurgical team and a hospital stay. Every patient in both arms is then followed for at least 36 months with clinic visits, rating scales, MRI scans and safety monitoring. Gene therapy rules also require long-term safety follow-up of treated patients after the primary endpoint. Around these per-patient activities sit fixed costs:

- a contract research organisation (CRO) that runs the trial;
- site start-up and contracting at perhaps 15 to 25 centres;
- an independent data safety monitoring board (DSMB);
- central MRI reading, biostatistics, regulatory reporting, and ex-US sites if they are used.

Evidence 1: uniQure's own spending

uniQure reports external program spending on AMT-130 every year. This "direct" research and development cost excludes staff, facilities and, until 2024, vector made in its own Lexington plant. The figures in US$ millions are [D901]–[D904]:

2017	2018	2019	2020	2021	2022	2023	2024	2025	H1 2026
3.9 [D901]	5.9 [D901]	4.1 [D901]	6.9 [D901]	10.5 [D901]	19.8 [D903]	13.0 [D902]	18.0 [D902]	42.5 [D902]	14.3 [D904]

The total is $138.9 million [I901]. Of that, 2025 includes $19.4 million to prepare the licence application and validate manufacturing [D902], and 2017–2018 was mostly pre-clinical and IND work (IND is the FDA filing needed to start human trials). That work will not recur.

The Phase I/II studies treated about 57 patients: 26 in the US, 13 in Europe, 12 in a third cohort and 6 in a fourth [D902][D903]. While patients were being dosed, the studies cost roughly $10–20 million a year. Excluding licence preparation and pre-IND work leaves about $110 million. That is close to $2 million per treated patient on a simple division [I902], but the figure overstates what a new patient costs. The trials have accumulated about 205 patient-years of follow-up. They ran for seven years across two countries and four cohorts, and the early US cohorts included imitation surgery. Allowing about $30 million for fixed overhead and $150,000 per patient-year of follow-up leaves roughly $0.85 million per dosing event, excluding vector [I903]. We treat that as the ceiling for the non-vector cost of treating one patient in a larger and more routine study.

uniQure also gives a funding statement. As of 30 June 2026 it held $811.9 million, which it expects to fund operations "into 2030, including costs associated with the commercial launches and confirmatory study of AMT-130" [D904]. We ran a simple cash walk from mid-2026 to the end of 2029 with these inputs [I904]:

- the Q2 2026 quarterly cost base (R&D $34.0 million, selling and administration $17.4 million) [D904];
- an allowance for launch spending;
- no AMT-130 sales.

With no sales, a study of about $150 million or less leaves cash roughly at zero by the end of 2029 once the $100 million Hercules loan tranche is drawn. A $250 million study leaves a shortfall of about $70 million. Any launch revenue from late 2027 closes that gap, so the statement leans slightly towards the lower half of our range but cannot rule out the upper half.

Evidence 2: Aspen Neuroscience's planned Phase 3

Aspen Neuroscience priced its initial public offering in October 2026 at $15.00 a share, for about $125.8 million of net proceeds [D905]. Its prospectus allocates "approximately $110.0 million to initiate and fund a Phase 3 trial" of sasineprocel, its cell therapy for Parkinson's disease (p.85) [D905]. Two further statements matter.

- **Runway:** the proceeds and existing cash will fund operations only "into the second half of 2028".
- **Completion:** they "will not be sufficient to complete... the Phase 3 trial" (p.85) [D905].

The trial will be "a single Phase 3 registrational study that includes a sham-controlled trial design and an 18-month primary endpoint" (p.131) [D906]. Aspen expects to start collecting patient biopsies by the end of 2026 and to dose patients in the second half of 2027 (p.4, p.113) [D906]. The prospectus does not give the number of patients. We assume about 100 [S901], in line with two comparable Parkinson's surgical trials described below. The $110 million therefore covers roughly the first two years of a trial that reads out around 2030. If it is half to two-thirds of the total, the full trial costs about $165–220 million, or $1.6–2.2 million per randomised patient [I905].

Scaling this to AMT-130 needs three adjustments.

- **Product.** Sasineprocel is made separately for each patient from their own skin cells (an autologous induced pluripotent stem cell product). That makes it far more expensive per patient than an off-the-shelf AAV vector. Aspen spent $14.5 million on "cell-therapy development and production" in 2025 [D907].
- **Control arm.** Aspen's controls receive sham surgery. AMT-130's controls have no surgery.
- **Size and length.** AMT-130's study is about twice the size and twice the length (36 versus 18 months).

We remove $0.5–1.0 million of manufacturing per treated patient and the sham-surgery cost, and add a longer follow-up and a vector cost. That gives $1.0–1.35 million per treated AMT-130 patient and $0.25–0.40 million per control. On 133 treated and 67 control patients, plus $30–40 million of fixed costs, the anchor implies roughly **$180–260 million** for AMT-130 [I906].

Evidence 3: the closest comparable trials

Bayer runs two Parkinson's surgical trials of similar size. Neither has a disclosed cost.

- **AskBio REGENERATE-PD:** a Phase 2 trial of AB-1005, an AAV gene therapy injected into the putamen. It is sham-surgery controlled, measures a motor diary at 18 months and runs at 46 sites. ClinicalTrials.gov lists 127 estimated participants, against 87 in AskBio's own release (NCT06285643) [D908].
- **BlueRock exPDite-2:** a Phase 3 trial of bemdaneprocel, a stem-cell-derived therapy. It has 102 estimated participants, a sham comparator, a 78-week endpoint and 43 sites, with primary completion expected in October 2028 (NCT06944522) [D909].

Bayer reports only portfolio-level figures. These are more than €3.5 billion invested in cell and gene therapy since 2020 and a $250 million Berkeley plant built partly to supply late-stage bemdaneprocel trials [D910]. These trials confirm that a sham-controlled neurosurgical study of about 100 patients needs 40–50 sites. They give no cost data. An AMT-130 study of 200 patients will therefore probably need 20 or more sites in more than one country.

Evidence 4: published benchmarks

- **Moore and colleagues (2018)** estimated the cost of pivotal trials for drugs the FDA approved in 2015–2016 [B901]. The median was $19.0 million, or $41,117 per patient. Trials with a placebo or active control averaged $35.1 million. These estimates exclude the cost of making the drug.
- **Sertkaya and colleagues (2016)**, for the US Department of Health and Human Services, found that Phase 3 studies average $11.5–52.9 million across therapeutic areas, with central nervous system studies among the cheapest [B902].
- **Health Advances** divided reported program costs by enrolment for orphan enzyme and gene therapies [B903]. It found "large costs per patient (~$2MM–~$5.7MM)", and a median for the largest trial of "~$100MM (range $42MM–$175MM)", for trials of 12–33 patients.

Conventional benchmarks therefore point to per-patient costs in the tens of thousands of dollars, and gene-therapy benchmarks point to millions. AMT-130's study sits between the two: an expensive one-off procedure followed by conventional follow-up.

The two largest procedure-specific inputs have partial sources.

- **ClearPoint hardware:** ClearPoint's chief executive said on 29 October 2026 that he expects "somewhere between $13,000 and $20,000 per procedure", with $15,000 a reasonable modelling assumption [D911].
- **Vector:** published estimates put AAV cost of goods at $0.5–1.0 million per dose generally [B904] and Hemgenix vector at $1–2 million per dose [B905]. Hemgenix is a high, whole-body dose. AMT-130 is a small local dose to the brain, so we use $50,000–300,000 per treated patient [I907].

Hospital, surgeon and imaging costs for the operation have no public source. We infer $0.2–0.85 million per treated patient, including site fees; the top of that range is the Phase I/II calibration above [I903].

Evidence 5: the CEO-simulation view

The simulation bot estimates the study at $190 million (low), $250 million (base) and $320 million (high) [K901]. Its base case phases spending as $50, 75, 62, 38 and 25 million over FY27–FY31. It scales from Phase I/II direct spend of about $2 million per patient to $1.2–1.8 million per treated patient. It puts controls at $0.3–0.5 million each and start-up at $15–30 million. We checked its historical spending figures against the filings and they are correct. As explained above, its per-patient rate does not remove the non-recurring Phase I/II overhead, so we regard its base case as nearer our upper band than our centre.

Cost bands

We combined the evidence in four steps.

- **Bottom-up build.** Per-patient costs for treated and control patients, plus fixed costs, give $84–275 million with $150 million in the middle [I908].
- **Aspen anchor.** This gives $180–260 million [I906].
- **CEO simulation.** This gives $190–320 million [K901].
- **Health Advances.** The largest-trial median of about $100 million (up to $175 million) is for much smaller trials [B903].

The runway statement leans slightly lower [I904]. We set the central estimate at **$190 million**, the point where the upper half of the bottom-up build meets the lower half of the Aspen anchor.

The **lower band of $120 million** assumes routine surgery at high-volume centres, a vector cost near $50,000, and lean follow-up. It sits above the Health Advances median, because AMT-130's study has six to fifteen times as many patients.

The **upper band of $290 million** assumes Phase I/II-level surgical cost, a vector cost near $300,000, many ex-US sites and slow enrolment. It approaches the simulation's high case.

Spending is phased by the trial's activity. Start-up begins in late 2026. Enrolment and surgery run through 2027–2028. Follow-up runs to 2030–2031. A long-term safety tail follows.

Figure 14 – Gross confirmatory-study cost by band and year (US$M)

US$ million	FY26	FY27	FY28	FY29	FY30	FY31	FY32	Total	FY27–31
Lower [I908]	3	25	35	26	17	9	5	120	112
Central [I908]	4	40	55	42	27	15	7	190	179
Upper [I908]	6	60	85	65	40	22	12	290	272

What goes on the forecast's confirmatory line

The forecast builds operating cost as the Q2 2026 run rate grown at 3% a year, with a separate line for the confirmatory study. The Q2 2026 run rate already contains $6.3 million a quarter, about $25 million a year, of AMT-130 external spending [D904]. That covers Phase I/II follow-up and licence-application work. As that work ends, we assume about $10 million a year of it is in effect redeployed to the confirmatory study, while the rest continues to fund long-term follow-up of Phase I/II patients [S902]. The separate line should therefore carry the gross cost less $10 million a year:

Figure 15 – Gross study cost, run-rate offset and separate confirmatory line (US$M)

US$M	FY27E	FY28E	FY29E	FY30E	FY31E	Total FY27E–FY31E
Lower: gross study cost [I908]	25	35	26	17	9	112
Lower: less run-rate offset [S902]	−10	−10	−10	−10	−9	−49
Lower: separate confirmatory line [I908][S902]	**15**	**25**	**16**	**7**	**0**	**63**
Central: gross study cost [I908]	40	55	42	27	15	179
Central: less run-rate offset [S902]	−10	−10	−10	−10	−10	−50
Central: separate confirmatory line [I908][S902]	**30**	**45**	**32**	**17**	**5**	**129**
Upper: gross study cost [I908]	60	85	65	40	22	272
Upper: less run-rate offset [S902]	−10	−10	−10	−10	−10	−50
Upper: separate confirmatory line [I908][S902]	**50**	**75**	**55**	**30**	**12**	**222**

Source: gross cost from the cost bands above [I908]; offset is our assumption [S902].

The roughly $4 million of FY26 start-up spend and the roughly $7 million FY32 tail are smaller than the $10 million a year offset, so they are treated as absorbed in the run rate and carry no separate line. Over FY27–FY31 the separate line totals $129 million, against $179 million of gross study cost in those years.

How the cost enters the forecast and the valuation

The central separate-line profile is carried unchanged in all three scenarios. In base and bull it is added to an opex build that starts from the Q2 2026 run rate; in bear it is added on top of the restructured opex (Figure A3). Bear still assumes AMT-130 is not approved, but the study starts screening before the FDA's decision and is assumed to be committed, so bear pays for it as well. Each scenario's EBITDA is therefore lower by exactly the amount on the line in each year from FY2027E to FY2031E, and is unaffected before and after (Figure 16).

Figure 16 – Separate confirmatory-study line and EBITDA with and without it (US$M)

US$M	FY27E	FY28E	FY29E	FY30E	FY31E
Confirmatory line, all scenarios [I1]	30	45	32	17	5
Base EBITDA excluding the study [C702]	309.8	3,764.1	4,625.5	4,683.0	4,146.5
Base EBITDA as forecast [D46]	279.8	3,719.1	4,593.5	4,666.0	4,141.5
Bull EBITDA excluding the study [C702]	1,503.4	7,791.3	6,759.3	7,001.3	7,105.1
Bull EBITDA as forecast [D46]	1,473.4	7,746.3	6,727.3	6,984.3	7,100.1
Bear EBITDA excluding the study [C702]	-141.8	-117.1	-102.4	-98.7	-95.4
Bear EBITDA as forecast [D46]	-171.8	-162.1	-134.4	-115.7	-100.4

Source: forecast file [D46]; "excluding" adds the line back [C702].

The cost bands were run through the same forecast and valuation pipeline, changing nothing but the confirmatory line, and resizing the bear year-0 equity raise in each run so that bear cash never goes negative [P109]. Figure 17 shows the result. Because approval is assumed in base and bull, a cost of this size barely moves their values: between the lower and upper bands, base value per share moves by about $1.28 (from $336.18 to $334.90), well under 1% of the central value. In bear, where there is no revenue to absorb it, the cost goes straight into the cash the company must raise: the raise is $4,150M on the lower band, $4,250M at the centre and $4,300M on the upper band. Bear value per share is close to zero in every band (-$1.02, -$0.78 and -$0.84). The ordering is not monotonic because the raise is sized in $50M steps, so the cash left over at the end differs between runs; in bear a higher study cost shows up mainly as a larger raise and more dilution rather than as a lower value per share.

Figure 17 – Value per share and bear raise by confirmatory-cost band

Cost profile (separate line, FY27E–FY31E total)	Base value per share	Bull value per share	Bear value per share	Bear year-0 raise [P109]	Bear minimum cash
Lower ($63M)	$336.18 [D47]	$561.20 [D47]	-$1.02 [D47]	$4,150M [D47]	9.59 [D47]
Central ($129M)	$335.65 [D45]	$560.67 [D45]	-$0.78 [D45]	$4,250M [D45]	43.59 [D45]
Upper ($222M)	$334.90 [D47]	$559.93 [D47]	-$0.84 [D47]	$4,300M [D47]	0.59 [D47]

Source: central run [D45]; lower and upper band runs [D47]. Each run uses the same market data, capex, demand model and 2% terminal growth; only the confirmatory line and the bear raise differ.

Verified versus assumed

- **Verified in primary documents:**
  - uniQure's yearly AMT-130 spending, its cash and runway statements, and the confirmatory design;
  - Aspen's use of proceeds, runway, Phase 3 design and timing, and R&D history;
  - the AskBio and BlueRock trial designs on ClinicalTrials.gov;
  - ClearPoint's per-procedure revenue.
- **Benchmarks:** the Moore, Sertkaya and Health Advances figures come from the publications or their abstracts. The Sertkaya central nervous system figure is from a summary, not the full table.
- **Not disclosed anywhere we could find:**
  - the size of Aspen's Phase 3 [S901];
  - a cost for any of the comparator trials;
  - AMT-130's cost of goods or Genezen pricing;

    - hospital and surgeon fees for the procedure;
    - a ClinicalTrials.gov listing for the confirmatory study, which was not posted as of 2 November 2026.
- **Inferences:** per-patient costs, the Aspen scaling, the phasing and the run-rate offset are our own [I901]–[I908], [S901][S902]. Confidence is low to moderate.
- **Largest single uncertainty:** the non-vector cost of each operation, which alone moves the total by about ±$60 million.
- **Next disclosure:** uniQure's Q3 2026 results on 4 November 2026 may narrow the range.

Appendix A – Bull and Bear Cases

The assumptions and line-by-line forecasts for the two alternative scenarios.

Bull. Incidence of 1.59 per 100,000, a 95% referral share and an 85.7% treated share; the same 6,000 start pool with unfiltered entrants (a steady pool of ~8,200) [C3]. Capacity is 880 / 4,500 / 3,500 and binds every year [S6]. Price is $2.4M, $2.16M from FY2028E [S10]. The UK launches from Q3 2028 at $1.5M [S8].

Figure A1 – Bull case by line

Row (units)	FY24A	FY25A	FY26E	FY27E	FY28E	FY29E	FY30E	FY31E	FY32E	FY33E
US centers implied by capacity at 2/week (count)	–	–	0 [C135]	17 [C136]	43 [C137]	34 [C138]	34 [C139]	34 [C140]	34 [C141]	34 [C142]
US treatment capacity (patients/yr)	–	–	0 [C143]	880 [C144]	4,500 [C145]	3,500 [C146]	3,500 [C147]	3,500 [C148]	3,500 [C149]	3,500 [C150]
US entrants to eligible pool (patients/yr)	–	–	0 [C151]	2,581 [C152]	5,187 [C153]	5,213 [C154]	5,239 [C155]	5,265 [C156]	5,292 [C157]	5,318 [C158]
US pool demand (patients/yr)	–	–	0 [C159]	11,196 [C160]	18,258 [C161]	12,651 [C162]	11,572 [C163]	11,099 [C164]	10,936 [C165]	10,931 [C166]
US untreated pool leavers (patients/yr)	–	–	0 [C167]	1,848 [C168]	3,013 [C169]	2,088 [C170]	1,910 [C171]	1,832 [C172]	1,805 [C173]	1,804 [C174]
US eligible pool at year end (patients)	–	–	6,000 [C175]	5,853 [C176]	3,527 [C177]	3,153 [C178]	2,982 [C179]	2,916 [C180]	2,903 [C181]	2,918 [C182]
US patients treated (patients/yr)	–	–	0 [C183]	880 [C184]	4,500 [C185]	3,500 [C186]	3,500 [C187]	3,500 [C188]	3,500 [C189]	3,500 [C190]
US cumulative treated (patients)	–	–	0 [C191]	880 [C192]	5,380 [C193]	8,880 [C194]	12,380 [C195]	15,880 [C196]	19,380 [C197]	22,880 [C198]
UK capacity (patients/yr)	–	–	0 [C199]	0 [C200]	117 [C201]	663 [C202]	780 [C203]	780 [C204]	524 [C205]	269 [C206]
UK patients treated (patients/yr)	–	–	0 [C207]	0 [C208]	117 [C209]	663 [C210]	780 [C211]	780 [C212]	524 [C213]	269 [C214]
UK eligible backlog at year end (patients)	–	–	0 [C215]	0 [C216]	2,155 [C217]	1,574 [C218]	943 [C219]	372 [C220]	0 [C221]	0 [C222]
License revenue, Hemgenix royalties (US$M)	10.1 [A1]	15.9 [A2]	24.9 [C8]	30.0 [S13]	35.0 [S14]	40.0 [S15]	44.0 [S16]	47.5 [S17]	50.0 [S18]	52.2 [S19]
Collaboration revenue (US$M)	10.9 [A3]	0.2 [A4]	0.0 [S20]	0.0 [S20]	0.0 [S20]	0.0 [S20]	0.0 [S20]	0.0 [S20]	0.0 [S20]	0.0 [S20]
Contract manufacturing revenue (US$M)	6.1 [A5]	0.0 [A25]	0.0 [G14]	0.0 [G14]	0.0 [G14]	0.0 [G14]	0.0 [G14]	0.0 [G14]	0.0 [G14]	0.0 [G14]
AMT-130 revenue, US (US$M)	0.0 [A26]	0.0 [A26]	0.0 [S21]	2,112 [C108]	9,720 [C109]	7,560 [C110]	7,560 [C110]	7,560 [C110]	7,560 [C110]	7,560 [C110]
AMT-130 revenue, UK (US$M)	0.0 [A26]	0.0 [A26]	0.0 [S34]	0.0 [S34]	176 [C111]	994 [C112]	1,170 [C113]	1,170 [C113]	787 [C114]	404 [C115]
Total revenue (US$M)	27.1 [A6]	16.1 [A7]	24.9 [C8]	2,142 [C117]	9,930 [C118]	8,594 [C119]	8,774 [C120]	8,778 [C121]	8,397 [C122]	8,016 [C123]
Gross margin (%)	–	89.5% [C32]	93.9% [C124]	87.6% [C125]	87.2% [C126]	87.9% [C127]	89.0% [C128]	90.2% [C129]	91.3% [C130]	91.5% [C131]
EBITDA (US$M)	-149 [C27]	-153 [C28]	-161 [C31]	1,473 [S35]	7,746 [S36]	6,727 [S37]	6,984 [S38]	7,100 [S39]	6,868 [S40]	6,546 [S41]
EBITDA margin (%, derived from the two rows above)	-550.9%	-949.9%	-645.9%	68.8%	78.0%	78.3%	79.6%	80.9%	81.8%	81.7%

Source: forecast file [D46]. me 81.7% [C134], mc 86.6% [S42], gsnext 0.02% [S43].

Bear. AMT-130 is not approved. Royalties continue and restructured opex falls from $170M in FY2027E to $140M from FY2029E. Bear also carries the same confirmatory-study line as base and bull, $30M, $45M, $32M, $17M and $5M in FY2027E–FY2031E [I1], shown as its own line in Figure A3 (Section 10 explains the estimate).

Figure A2 – Bear case by line

Row (units)	FY24A	FY25A	FY26E	FY27E	FY28E	FY29E	FY30E	FY31E	FY32E	FY33E
License revenue, Hemgenix royalties (US$M)	10.1 [A1]	15.9 [A2]	24.9 [C8]	30.0 [S13]	35.0 [S14]	40.0 [S15]	44.0 [S16]	47.5 [S17]	50.0 [S18]	52.2 [S19]
Collaboration revenue (US$M)	10.9 [A3]	0.2 [A4]	0.0 [S20]	0.0 [S20]	0.0 [S20]	0.0 [S20]	0.0 [S20]	0.0 [S20]	0.0 [S20]	0.0 [S20]
Contract manufacturing revenue (US$M)	6.1 [A5]	0.0 [A25]	0.0 [G14]	0.0 [G14]	0.0 [G14]	0.0 [G14]	0.0 [G14]	0.0 [G14]	0.0 [G14]	0.0 [G14]
AMT-130 revenue, US (US$M)	0.0 [A26]	0.0 [A26]	0.0 [S44]	0.0 [S44]	0.0 [S44]	0.0 [S44]	0.0 [S44]	0.0 [S44]	0.0 [S44]	0.0 [S44]
AMT-130 revenue, UK (US$M)	0.0 [A26]	0.0 [A26]	0.0 [S45]	0.0 [S45]	0.0 [S45]	0.0 [S45]	0.0 [S45]	0.0 [S45]	0.0 [S45]	0.0 [S45]
Total revenue (US$M)	27.1 [A6]	16.1 [A7]	24.9 [C8]	30.0 [C223]	35.0 [C224]	40.0 [C225]	44.0 [C226]	47.5 [C227]	50.0 [C228]	52.2 [C229]
Gross margin (%)	–	89.5% [C32]	93.9% [C230]	93.9% [C230]	93.9% [C230]	93.9% [C230]	93.9% [C230]	93.9% [C230]	93.9% [C230]	93.9% [C230]
EBITDA (US$M)	-149 [C27]	-153 [C28]	-161 [C31]	-172 [S47]	-162 [S48]	-134 [S49]	-116 [S50]	-100 [S51]	-93.0 [S52]	-90.9 [S53]
EBITDA margin (%, derived from the two rows above)	-550.9%	-949.9%	-645.9%	-572.7%	-463.2%	-336.1%	-262.9%	-211.3%	-186.1%	-174.0%

Source: forecast file [D46]. me -174.0% [C233], mc 93.9% [S54], gsnext 3.00% [S55].

Figure A3 – Bear cash opex build (US$M)

Row (US$M)	FY27E	FY28E	FY29E	FY30E	FY31E	FY32E	FY33E
Restructured opex [S403]	170	150	140	140	140	140	140
Confirmatory study [I1]	30	45	32	17	5	0	0
Bear cash opex	200	195	172	157	145	140	140
Bear EBITDA	-172 [S47]	-162 [S48]	-134 [S49]	-116 [S50]	-100 [S51]	-93.0 [S52]	-90.9 [S53]

Source: forecast file [D46]; restructured opex is our assumption [S403]; confirmatory study [I1]; EBITDA from Figure A2.

Appendix B – Numbered sources

Every tag cited in the report, grouped by type. File paths identify the data files the numbers were read from.

Documents

- **D1** uniQure Form 10-K FY2025 (filed 2026-03-02). https://www.sec.gov/Archives/edgar/data/1590560/000110465926021898/qure-20251231x10k.htm. HTML; no page numbers:
- **D2** uniQure Form 10-Q Q2 2026 (filed 2026-07-29). https://www.sec.gov/Archives/edgar/data/1590560/000110465926087815/qure-20260630x10q.htm. HTML; no page numbers:
- **D3** uniQure Q2 2026 results press release, 8-K Ex.99.1 (2026-07-29). https://www.sec.gov/Archives/edgar/data/1590560/000110465926087808/qure-20260729xex99d1.htm. HTML; no page numbers:
- **D4** uniQure BLA submission press release, 8-K Ex.99.1 (2026-09-02). https://www.sec.gov/Archives/edgar/data/1590560/000110465926104463/qure-20260902xex99d1.htm. HTML; no page numbers:

- **D5** uniQure four-year AMT-130 data press release, 8-K Ex.99.2 (2026-09-29). https://www.sec.gov/Archives/edgar/data/1590560/000110465926111654/tm2626514d1_ex99-2.htm. HTML; no page numbers:
- **D6** Yahoo Finance / yfinance analyst revenue estimates for QURE, pulled 2026-10-29 (shown in EUR). https://finance.yahoo.com/quote/QURE/analysis/. HTML; no page numbers:
- **D7** GEN StockWatch, Leerink net-price estimates and O'Keefe pricing/TAM comments (2025). https://www.genengnews.com/topics/translational-medicine/stockwatch-uniqure-shares-reach-five-year-high-on-game-changing-huntingtons-data/. HTML; no page numbers:
- **D8** Clinaptis research, AMT-130 capacity ramp and net-price model. https://www.clinaptisresearch.com/research/amt-130-huntingtons-uniqure-bla. HTML; no page numbers:
- **D9** BioSpace, uniQure files for FDA, UK approval (2026-09). https://www.biospace.com/fda/uniqure-files-for-fda-uk-approval-of-first-gene-therapy-for-huntingtons-disease. HTML; no page numbers:
- **D10** uniQure, Pivotal Phase I/II AMT-130 Huntington's Disease Update, investor presentation, slides 25-26 (2025-09-24). https://cdn.craft.cloud/fe68e1a2-b284-4f69-ba29-921f065edf17/assets/uploads/AMT-130-Pivotal-Sept-2025_final_09.24.25.pdf. HTML; no page numbers:
- **D11** Medina et al., Prevalence and Incidence of Huntington's Disease: An Updated Systematic Review and Meta-Analysis, Movement Disorders 2022. https://pmc.ncbi.nlm.nih.gov/articles/PMC10086981/. HTML; no page numbers:
- **D12** uniQure Q2 2026 earnings call transcript (Motley Fool, 2026-08-07). https://www.fool.com/earnings/call-transcripts/2026/08/07/uniqure-qure-q2-2026-earnings-call-transcript/. HTML; no page numbers:
- **D13** uniQure Q1 2026 earnings call transcript (StockAnalysis). https://stockanalysis.com/stocks/qure/transcripts/578165-q1-2026/. HTML; no page numbers:
- **D17** Analyst instructions, dated 2026-10-30 (capacity-limited in the short term; yearly refill equals incidence less patients excluded for eligibility, willingness and coverage). Written instruction; no page numbers:
- **D22** Investing.com, Goldman Sachs note summary on uniQure (2025-09). https://ph.investing.com/news/analyst-ratings/uniqure-stock-rises-after-positive-huntingtons-disease-gene-therapy-data-93CH-2010942. HTML; no page numbers:
- **D23** Reuters, FDA approves Sarepta gene therapy (2023-06-22): Hemgenix list price. https://www.reuters.com/business/healthcare-pharmaceuticals/us-fda-approves-sareptas-gene-therapy-rare-muscular-dystrophy-some-kids-2023-06-22/. HTML; no page numbers:
- **D24** ClearPoint Neuro Q2 2026 earnings call transcript (2026-08-10, Motley Fool): CLEAR site-readiness program. https://www.fool.com/earnings/call-transcripts/2026/08/10/clearpoint-neuro-clpt-q2-2026-earnings-call-transcript/. HTML; no page numbers:
- **D26** ClearPoint Neuro Q3 2026 earnings call transcript (2026-10-28, StockAnalysis). https://stockanalysis.com/stocks/clpt/transcripts/722761-q3-2026/. HTML; no page numbers:
- **D28** IntuitionLabs, Most Expensive Drugs in the US: 2025 price list (secondary source for gene-therapy list prices). https://intuitionlabs.ai/articles/most-expensive-drugs-usa-2025. HTML; no page numbers:
- **D31** Analyst instructions, dated 2026-10-30 (pool model; leave rate m = 0.62; valuation run). Written instruction; no page numbers:
- **D32** Forecaster CLPT case, forecasts/CLPT/forecast.md (AMT-130 capacity ramp 330 / 2,500 / 3,000 base, 880 / 4,500 / 3,500 bull) [S], uncommitted local case. n/a: local forecaster case file /workspace/forecaster-work/forecaster-main/forecasts/CLPT/forecast.md. HTML; no page numbers:
- **D33** uniQure, update on Phase I/II AMT-130 trials: 4 of 6 high-dose controls crossed over, 2 failed inclusion criteria (link unverified, search result). https://uniqure.gcs-web.com/news-releases/news-release-details/uniqure-announces-update-phase-iii-clinical-trials-amt-130-gene. HTML; no page numbers:
- **D45** Finagle valuation result: /workspace/out/QURE_run_v14/valuation_result_v14.json, case_{base,bull,bear}*v14.json, QURE*{base,bull,bear}_v14.xlsx (Finagle commit 377f177b, run 2026-11-02T11:40:44.828755-05:00)
- **D46** Forecast file /workspace/out/QURE_run_v14/forecast.json and valuation request /workspace/out/QURE_run_v14/valuation_request_v14.json (file sha256 5c64a274…8425; whole request 5089ab65…8299)
- **D47** Band runs of the same pipeline: /workspace/out/QURE_run_v14_band_lower/valuation_result_v14.json and /workspace/out/QURE_run_v14_band_upper/valuation_result_v14.json (Finagle commit 377f177b)
- **D48** uniQure 10-K FY2025, consolidated balance sheets (page number not captured): "Cash and cash equivalents $ 80,240 … Current investment securities 542,301" (US$ thousands, 31 December 2025). https://www.sec.gov/Archives/edgar/data/1590560/000110465926021898/qure-20251231x10k.htm

- **D49** uniQure 8-K dated 24 June 2026, Item 1.01: "On June 24, 2026, the Underwriters exercised the Option in full. The Company estimates that the net proceeds from the Offering will be approximately $242.7 million after deducting underwriting discounts and commissions and estimated offering expenses." https://www.sec.gov/Archives/edgar/data/1590560/000110465926077339/tm2618105d1_8k.htm
- **D50** uniQure 10-Q Q2 2026, condensed consolidated statements of shareholders' equity (page number not captured): "Follow-on public offering 5,686,813 ... 242,681" (shares; US$ thousands). Employee option exercises and ESPP added $6.985M in H1 2026. https://www.sec.gov/Archives/edgar/data/1590560/000110465926087815/qure-20260630x10q.htm
- **D601** GEN StockWatch, 28 Sept 2025. Paraphrase: "About 6,000 patients, representing the initially treatable ~30% of the total ~20,000." https://www.genengnews.com/topics/translational-medicine/stockwatch-uniqure-shares-reach-five-year-high-on-game-changing-huntingtons-data/
- **D602** ClinicalTrials.gov NCT04120493: early manifest HD, TFC 9 to 13, ages 25 to 65, CAG 40 or more, minimum putamen and caudate volumes. https://clinicaltrials.gov/study/NCT04120493
- **D603** Medscape, 2026: the two other controls "no longer met eligibility criteria." https://www.medscape.com/viewarticle/huntington-therapy-faces-regulatory-roller-coaster-2026a1000k0l (search-result snippet)
- **D604** EHDN Platform Meeting report, Nov 2025: "four were eligible and accepted the offer." https://ehdn.org/wp-content/uploads/2025/12/EHDN-Platform-Meeting-uniQure-Topline-Results-Report_Final_2-04-Dec-2025.pdf (search-result snippet)
- **D605** Quarterly stock-and-flow pool structure, as used in the ClearPoint (CLPT) forecast; model files /workspace/out/AMT130_demand/scratch_m62/
- **D606** ClearPoint Neuro CEO capacity comments, Q2 2026 call, as used in the CLPT forecast
- **D607** Secondary review of uniQure filings and call transcripts (simulated-management research tool built on public sources; not an official uniQure statement): no treat-target language found; criteria behind the ~30% undisclosed
- **D608** Wexler NS et al., Incidence of adult Huntington's disease in the UK: a UK-based primary care study and a systematic review, BMJ Open 2016;6:e009070: "overall rate of 7.2 (95% CI 6.5 to 7.9) per million person-years"; 393 incident cases, CPRD, adults 21+, 1990-2010. https://pmc.ncbi.nlm.nih.gov/articles/PMC4769413/ (abstract and methods only)
- **D901** uniQure 10-K FY2019, MD&A R&D table: "Huntington's disease (AMT-130) 4,126 5,862 3,922" (2019/2018/2017, $000). https://www.sec.gov/Archives/edgar/data/1590560/000155837020001883/qure-20191231x10k5849ab.htm ; 10-K FY2021: "Huntington's disease (AMT-130) 10,529 6,905 4,126". https://www.sec.gov/Archives/edgar/data/1590560/000155837022002050/qure-20211231x10k.htm
- **D902** uniQure 10-K FY2025, MD&A (p.~99–100): "$42.5 million, $18.0 million and $13.0 million... includes $19.4 million of costs incurred in relation to the preparation of the BLA submission". https://www.sec.gov/Archives/edgar/data/1590560/000110465926021898/qure-20251231x10k.htm
- **D903** uniQure 10-K FY2023 (p.~92): "We enrolled 26 patients into our U.S. clinical trial... and enrolled 13 patients into our European clinical trial". https://www.sec.gov/Archives/edgar/data/1590560/000155837024001915/qure-20231231x10k.htm The FY2022 AMT-130 direct R&D of $19.8M is read from the same 10-K MD&A R&D table; the exact cell quote was not captured (partly unverified).
- **D904** uniQure 10-Q Q2 2026 (p.25, 29 and liquidity section): AMT-130 "$6.3 million" (Q2), "$14.3 million" (H1); "sufficient to fund our projected operating expenses into 2030, including costs associated with the commercial launches and confirmatory study of AMT-130". https://www.sec.gov/Archives/edgar/data/1590560/000110465926087815/qure-20260630x10q.htm
- **D905** Aspen Neuroscience 424B4, Use of Proceeds (p.85): "net proceeds to us from this offering will be approximately $125.8 million (or approximately $145.4 million if the underwriters exercise their option to purchase additional shares in full)"; "approximately $110.0 million to initiate and fund a Phase 3 trial evaluating sasineprocel"; "sufficient to fund our operations into the second half of 2028"; "will not be sufficient to complete the Phase 1/2a ASPIRO trial, the Phase 3 trial". https://www.sec.gov/Archives/edgar/data/2124080/000119312526441782/elevation_424b4.htm (S-1: https://www.sec.gov/Archives/edgar/data/2124080/000119312526419104/na-20261009.htm)
- **D906** Aspen Neuroscience 424B4 (same link as D905), p.131: "a single Phase 3 registrational study that includes a sham-controlled trial design and an 18-month primary endpoint"; p.4 and p.113: "We anticipate initiating biopsy collection for the Phase 3 trial by the end of 2026... dosing patients in the Phase 3 trial in the second half of 2027."
- **D907** Aspen Neuroscience 424B4 (same link as D905), MD&A (p.93–95): R&D "$41.8 million... 2025, compared to $35.6 million... 2024"; "Cell-therapy development and production costs $14,497"; H1 2026 R&D $28.5 million. Series C (p.92): "gross proceeds of approximately $115.6 million".
- **D908** ClinicalTrials.gov NCT06285643 (API v2): Phase 2, enrolment 127 (estimated), arms AAV2-GDNF vs "Control Surgery: SHAM_COMPARATOR", primary endpoint PD Motor Diary at 18 months, 46 locations. AskBio release: "an estimated 87 participants". https://clinicaltrials.gov/study/NCT06285643 ; https://www.askbio.com/first-european-participants-randomized-in-askbio-phase-2-gene-therapy-trial-of-ab-1005-for-parkinsons-disease/ (web page; no page numbers)

- **D909** ClinicalTrials.gov NCT06944522 (API v2): Phase 3, enrolment 102 (estimated), sham comparator, primary endpoint at Week 78, 43 locations, primary completion October 2028. https://clinicaltrials.gov/study/NCT06944522 (web page; no page numbers)
- **D910** Bayer, 10 Oct 2023: "$250 million... facility will supply the material required for late-stage clinical trials and potential commercial launch of... bemdaneprocel". https://www.bayer.com/media/en-us/bayer-invests-250-million-usd-in-new-cell-therapy-manufacturing-facility-in-the-us/ ; >€3.5 billion since 2020 (reported by VCBeat). https://www.vcbeathealth.com/article/16476 (web page; no page numbers)

- **D911** ClearPoint Neuro Q3 2026 earnings call, 29 Oct 2026: "somewhere between $13,000 and $20,000 per procedure... $15,000 would be a non-noisy assumption." https://www.fool.com/earnings/call-transcripts/2026/10/29/clearpoint-neuro-clpt-q3-2026-earnings-call-transcript/ (web page; no page numbers) **Actuals**

- **A1** License revenues $ 15,934 $ 10,133 $ 2,758 (D1, unverified)

- **A2** License revenues $ 15,934 $ 10,133 $ 2,758 (D1, unverified)
- **A3** Collaboration revenues 164 10,872 2,250 (D1, unverified)
- **A4** Collaboration revenues 164 10,872 2,250 (D1, unverified)
- **A5** Contract manufacturing revenues — 6,114 10,835 (D1, unverified)
- **A6** Total revenues $ 16,098 $ 27,119 $ 15,843 (D1, unverified)
- **A7** Total revenues $ 16,098 $ 27,119 $ 15,843 (D1, unverified)
- **A13** Lenmeldy holds the highest list price of any U.S. drug at $4.25 million per dose, followed by Kebilidi at $3.95 million. (D28, unverified)
- **A14** The Purchaser will receive 1.85 times the upfront payment (or $ 693.8 million) until June 30, 2032 (“First Hard Cap Date”) if such thresholds are met or, if such cap is not met by June 30, 2032, up to 2.25 times of the upfront payment through December 31, 2038 ... Liability from royalty financing agreement 473,199 (D1, unverified)
- **A16** The pooled incidence calculated from these studies was 0.48 cases per 100,000 person-years ... North America (1.04 per 100,000 person-years; 95% CI, 0.28-1.80) (D11, unverified)
- **A21** a gene therapy for bleeding disorder hemophilia from Australian drugmaker CSL Ltd ... carries a list price of $3.5 million. (D23, unverified)
- **A22** R&D expenses for the three months ended June 30, 2026 were $34.0 million ... Selling, general and administrative expenses for the three months ended June 30, 2026 were $17.4 million ... Net cash used in operating activities was $66.8 million for the six months ended June 30, 2026 ... cash and cash equivalents and investment securities of $810.3 million ... sufficient to fund our projected operating expenses into 2030 (D2, unverified)
- **A23** Four of the six control patients in the high-dose cohort were crossed over to treatment and the remaining two patients failed to meet the study's inclusion criteria. (D33, unverified)
- **A25** Contract manufacturing revenues — 6,114 (D1, unverified)

- **A26** investigational gene therapy (D1, unverified) **Guidance and management statements**

- **G1** I think we already have 15 centers just in the first couple months that have filled out and sort of qualified by saying, yes, not only do we have the materials, the patient recruitment, the cooperation with pharmacy to thaw out the drugs, all those types of things, but they've also pretty much raised their hand and say, yes, we could be ready to do 1, if not 2 of these procedures a week. So, if we can get to 20 or 30 of these centers that are each willing to commit 1 to 2 cases a week, which is 50 to 100 cases a week, you can imagine getting to a ramped-up procedure type pretty quickly here. (D24, unverified)

- **G3** the expectation would be a final formal decision sometime six to eight months after the acceptance of that BLA. Again, if everything went perfectly. That would put a potential approval for the very first neuro gene therapy to treat Huntington's disease sometime middle to second half of next year. (D26, unverified)
- **G4** Management and the FDA reached alignment on a randomized standard-of-care controlled design, substituting the previous sham procedure, with total functional capacity at 36 months as the primary endpoint ... the confirmatory study should be well underway and potentially fully enrolled at the time of accelerated approval. (D12, unverified)
- **G5** ~6,000 patients *Initial Treatable Patients at Launch ... ~20,000 patients Diagnosed HD ~40,000 patients Symptomatic HD ~100,000 patients Genetically Identifiable HD ...* Management Estimate. (slide 25) (D10, unverified)

- **G6** About 6,000 patients, representing the initially treatable ~30% of the total ~20,000 total diagnosed and symptomatic HD patients in the United States today. Approximately 40,000 U.S. patients are symptomatic for HD. (D7, unverified)
- **G9** There are a number of centers that are available to do this, and we are working with what we think is the right number in the initial term, and then we will continue to build from there. From a capacity point of view, it's very challenging to give you one number, and we're still working through that because it depends on the number of neurosurgeons at a particular hospital. It depends on the number of intraoperative OR suites and other competing priorities. (D12, unverified)
- **G12** its Marketing Authorisation Application (MAA) for ifezuntirgene inilparvovec has been submitted to the United Kingdom's (U.K.) Medicines and Healthcare products Regulatory Agency (MHRA) ... The Company has requested priority review for the BLA. If granted, priority review would shorten the FDA review cycle to six months following the FDA's 60-day BLA filing review period. (D4, unverified)
- **G13** we rely on Genezen for the production of HEMGENIX and will have preferential access to the Lexington Facility for the production of materials related to AMT-130 and AMT-191 programs under separately negotiated development and supply arrangements. (D1, unverified)
- **G14** divestment of our commercial manufacturing activities in July 2024 (D1, unverified)
- **G601** uniQure AMT-130 pivotal deck, 24 Sept 2025, slides 25 to 26: "~6,000* Initial Treatable Patients at Launch ... *Management Estimate". https://cdn.craft.cloud/fe68e1a2-b284-4f69-ba29-921f065edf17/assets/uploads/AMT-130-Pivotal-Sept-2025_final_09.24.25.pdf
- **G602** uniQure 424B5 prospectus, 24 Sept 2025: "We estimate there will be 6,000 initial treatable patients addressable by AMT-130 in the U.S. at commercial launch, assuming approval." https://www.sec.gov/Archives/edgar/data/1590560/000110465925092995/tm2526258-1_424b5.htm (not reopened, unverified)
- **G603** uniQure Q2 2026 call, 29 Jul 2026, K. O'Keefe: capacity "depends on the number of neurosurgeons ... intraoperative OR suites"; more detail "in the coming months."
- **G604** uniQure release on the Phase III update: "Four of the six control patients in the high-dose cohort were crossed over to treatment and the remaining two patients failed to meet the study's inclusion criteria." https://uniqure.gcs-web.com/news-releases/news-release-details/uniqure-announces-update-phase-iii-clinical-trials-amt-130-gene (search-result snippet)

- **G701** uniQure press release, 2 Nov 2026, "uniQure Announces FDA Acceptance and Priority Review of the Biologics License Application for ifezuntirgene inilparvovec for the Treatment of Huntington's Disease" (opening paragraphs and confirmatory-study paragraph): "the U.S. Food and Drug Administration (FDA) has accepted for review the Biologics License Application (BLA) for ifezuntirgene inilparvovec ... submitted under the Accelerated Approval pathway. The application has been granted Priority Review with a Prescription Drug User Fee Act (PDUFA) target action date of May 1, 2027." "the FDA stated that they are tentatively planning to discuss this application at an Advisory Committee meeting." "The study is designed as a randomized, standard-of-care trial enrolling approximately 200 patients in a 2:1 ratio ... with Total Functional Capacity (TFC) as the primary endpoint at 36 months. The Company plans to initiate patient screening in the U.S. by the end of 2026." "the Phase I/II clinical studies have dosed 51 patients". The BLA is supported by 36-month Phase I/II data vs a propensity score-matched Enroll-HD external control. https://www.globenewswire.com/news-release/2026/11/02/3393020/0/en/uniqure-announces-fda-acceptance-and-priority-review-of-the-biologics-license-application-for-ifezuntirgene-inilparvovec-for-the-treatment-of-huntington-s-disease.html (opened 2026-11-02; also https://uniqure.gcs-web.com/news-releases/news-release-details/uniqure-announces-fda-acceptance-and-priority-review-biologics, not opened) **Estimates**

- **E6** Goldman Sachs modeling potential peak global sales of $2.5 billion with a 90% probability of success, assuming approximately 25% cumulative penetration into the estimated 20,000 diagnosed, symptomatic patients in the US. (D22, unverified)

- **E601** 2CARE analysis: stay of about 2.3 to 2.9 years in the early-manifest window (paper not reopened, unverified)
- **E602** Movement Disorders, 2024: early-manifest stay of 2 to 3 years (paper not reopened, unverified)
- **E603** E070 dataset: 29-month mean, censored (not reopened, unverified)
- **E604** Marder et al., Neurology 2000: about 0.97 TFC points a year. https://pubmed.ncbi.nlm.nih.gov/10668713/ (snippet only, unverified)
- **E605** HDSA HD Roadmap: Stage I about 0 to 8 years. https://hdsa.org/wp-content/uploads/2015/07/HD-Roadmap2.pdf (snippet only, unverified)

- **E701** Reuters via WHBL, 2 Nov 2026, "FDA to review uniQure's Huntington's gene therapy, decision due on May 2027": 51 patients treated across studies. https://whbl.com/2026/11/02/fda-to-review-uniqures-huntingtons-gene-therapy-decision-due-on-may-2027/ (not opened, unverified) **Analyst instructions**

- **P8** Analyst instruction, dated 2026-10-30: run the Finagle valuation on this forecast (D31, unverified)

- **P9** Analyst instruction, dated 2026-10-30: use the pool model with m = 0.62, described as "the most internally consistent" leave rate (D31, unverified)
- **P101** Analyst instruction, dated 2026-10-29: "take rd [cost of debt] as 10%"
- **P103** Analyst instruction, dated 2026-10-29: "keep debt constant until the year when EBITDA is positive. Then target 1x EBITDA from that year onward."
- **P104** Analyst instruction, dated 2026-10-30: "Stock comp is held constant until EBITDA is positive and then is a fixed % of sales." Opening NOLs to be taken from the review of company filings.
- **P105** Analyst instruction, dated 2026-10-30: "Take depreciation as half of capex in year 0. Liability isn't debt. Yes to 1xebitda rule. Add shares and cash in year 0 so cash doesn't go negative anywhere. 2% terminal growth. Dutch tax rate."
- **P106** Analyst instruction, dated 2026-10-30: "use method=1 for capex. Capex can start in year 0 per [management] guidance but use the method after that."
- **P107** Analyst instruction, dated 2026-10-30: "Take the exact same capex as the base case for the bear case."
- **P109** Analyst instruction, dated 2026-10-30: "Raise the bear year-0 equity so cash never goes negative"
- **P110** Analyst instruction, dated 2026-11-02: "Check the Aspen s-1 for more data. They will run a trial similar in size to AskBio and BlueRock. So this gives another data point. Bull, Base and Bear can use the same cost but include a detailed section in the report explaining analysis and upper and lower cost bands. Make it self contained and use prose to explain."

- **P111** Analyst instruction, dated 2026-11-02: "Year-0 should be 2025 cash+short term. And then adjust for any equity raises during the 2026 year. Then increase sharecount from 2025 to account for the equity raises." **Assumptions**

- **S2** base treated share of the eligible pool P = referral 90% x willing 90% x covered 90% = 0.729 [S under P3] (D17, unverified)

- **S4** US HD incidence per 100k/yr used in the pool model: base 1.22, bull 1.59 (AMT130 pool model inputs; North America 1.04, 95% CI 0.28-1.80, in Medina 2022) [S] (D11, unverified)
- **S5** base US capacity = CLPT AMT-130 ramp 330 (FY27, from 2027-07-01) / 2500 (FY28) / 3000 per year cap from FY29, spread evenly by quarter; treated = min(capacity, demand). Centers shown = capacity / (2 procedures/week x 52), used only for the $1.5M/center opex item [S] (D32, unverified)
- **S6** bull US capacity = CLPT AMT-130 ramp 880 (FY27, from 2027-07-01) / 4500 (FY28) / 3500 per year cap from FY29, spread evenly by quarter; treated = min(capacity, demand). Centers shown = capacity / (2 procedures/week x 52), used only for the $1.5M/center opex item [S] (D32, unverified)
- **S8** Bull UK: MHRA approval and NHS access from Q3 2028; initial backlog 30% x 7,500 = 2250; refill 0.48/100k x 69M x bull haircut, 10%/yr attrition; centers [3, 6, 9, 12, 15]; 1 procedure/center/week (4.33/month); net price $1.5M [S] (D13, unverified)
- **S9** Bull UK: MHRA approval and NHS access from Q3 2028; initial backlog 30% x 7,500 = 2250; refill 0.48/100k x 69M x bull haircut, 10%/yr attrition; centers [3, 6, 9, 12, 15]; 1 procedure/center/week (4.33/month); net price $1.5M [S] (D7, unverified)
- **S10** Bull US net price $2.4M, then -10%% to $2.16M from the year after annual US sales first pass $2B [S] (D8, unverified)
- **S11** base AMT-130 COGS (Genezen manufacturing, release, cold-chain; no licensor royalty, none disclosed for AMT-130): $0.30M/patient in FY27, falling $0.020M/yr to $0.20M; UNVERIFIED, no uniQure disclosure [S] (D1, unverified)
- **S13** Hemgenix royalty build (US$M, GAAP license revenue): growth slows as Hemgenix matures; FY32+ +2.5/yr fading 5%/yr. Cash goes to HemB SPV until the royalty-financing cap is met (liability $473.2M = debt) [S] (D2, unverified)
- **S14** Hemgenix royalty build (US$M, GAAP license revenue): growth slows as Hemgenix matures; FY32+ +2.5/yr fading 5%/yr. Cash goes to HemB SPV until the royalty-financing cap is met (liability $473.2M = debt) [S] (D2, unverified)
- **S15** Hemgenix royalty build (US$M, GAAP license revenue): growth slows as Hemgenix matures; FY32+ +2.5/yr fading 5%/yr. Cash goes to HemB SPV until the royalty-financing cap is met (liability $473.2M = debt) [S] (D2, unverified)
- **S16** Hemgenix royalty build (US$M, GAAP license revenue): growth slows as Hemgenix matures; FY32+ +2.5/yr fading 5%/yr. Cash goes to HemB SPV until the royalty-financing cap is met (liability $473.2M = debt) [S] (D2, unverified)
- **S17** Hemgenix royalty build (US$M, GAAP license revenue): growth slows as Hemgenix matures; FY32+ +2.5/yr fading 5%/yr. Cash goes to HemB SPV until the royalty-financing cap is met (liability $473.2M = debt) [S] (D2, unverified)

- **S18** Hemgenix royalty build (US$M, GAAP license revenue): growth slows as Hemgenix matures; FY32+ +2.5/yr fading 5%/yr. Cash goes to HemB SPV until the royalty-financing cap is met (liability $473.2M = debt) [S] (D2, unverified)
- **S19** Hemgenix royalty build (US$M, GAAP license revenue): growth slows as Hemgenix matures; FY32+ +2.5/yr fading 5%/yr. Cash goes to HemB SPV until the royalty-financing cap is met (liability $473.2M = debt) [S] (D2, unverified)
- **S20** Collaboration services to CSL wound down after the 2024 Lexington Transaction; none in H1 2026 [S] (D1, unverified)
- **S21** No AMT-130 US revenue: no approval before 2027. (D9, unverified)
- **S22** No AMT-130 UK revenue in base (D4, unverified)
- **S23** base EBITDA (US$M) = license GP + AMT-130 revenue - COGS 99.0 - cash opex 309.3 (core Q2 2026 run rate x4 +3%/yr, US commercial fixed $80M, $1.5M per active center, 5% variable) [S] (D2, unverified)
- **S24** base EBITDA (US$M) = license GP + AMT-130 revenue - COGS 700.0 - cash opex 613.8 (core Q2 2026 run rate x4 +3%/yr, US commercial fixed $80M, $1.5M per active center, 5% variable) [S] (D2, unverified)
- **S25** base EBITDA (US$M) = license GP + AMT-130 revenue - COGS 780.0 - cash opex 664.1 (core Q2 2026 run rate x4 +3%/yr, US commercial fixed $80M, $1.5M per active center, 5% variable) [S] (D2, unverified)
- **S26** base EBITDA (US$M) = license GP + AMT-130 revenue - COGS 720.0 - cash opex 655.3 (core Q2 2026 run rate x4 +3%/yr, US commercial fixed $80M, $1.5M per active center, 5% variable) [S] (D2, unverified)
- **S27** base EBITDA (US$M) = license GP + AMT-130 revenue - COGS 656.6 - cash opex 618.4 (core Q2 2026 run rate x4 +3%/yr, US commercial fixed $80M, $1.5M per active center, 5% variable) [S] (D2, unverified)
- **S28** base EBITDA (US$M) = license GP + AMT-130 revenue - COGS 568.0 - cash opex 607.0 (core Q2 2026 run rate x4 +3%/yr, US commercial fixed $80M, $1.5M per active center, 5% variable) [S] (D2, unverified)
- **S29** base EBITDA (US$M) = license GP + AMT-130 revenue - COGS 566.3 - cash opex 613.1 (core Q2 2026 run rate x4 +3%/yr, US commercial fixed $80M, $1.5M per active center, 5% variable) [S] (D2, unverified)
- **S30** Average of me and FY33 gross margin (Finagle default rule); no positive-EBITDA history to set it against [S] (D2, unverified)
- **S31** FY39 growth: royalty 53.8 (+3%) + AMT-130 at the FY33 steady-state flow and price, vs FY33 revenue [S] (D8, unverified)
- **S34** No AMT-130 UK revenue in bull (UK launch from Q3 2028 in bull only) (D4, unverified)
- **S35** bull EBITDA (US$M) = license GP + AMT-130 revenue - COGS 264.0 - cash opex 402.8 (core Q2 2026 run rate x4 +3%/yr, US commercial fixed $90M, $1.5M per active center, 5% variable, UK $15M from FY28) [S] (D2, unverified)
- **S36** bull EBITDA (US$M) = license GP + AMT-130 revenue - COGS 1269.7 - cash opex 912.4 (core Q2 2026 run rate x4 +3%/yr, US commercial fixed $90M, $1.5M per active center, 5% variable, UK $15M from FY28) [S] (D2, unverified)
- **S37** bull EBITDA (US$M) = license GP + AMT-130 revenue - COGS 1040.8 - cash opex 824.0 (core Q2 2026 run rate x4 +3%/yr, US commercial fixed $90M, $1.5M per active center, 5% variable, UK $15M from FY28) [S] (D2, unverified)
- **S38** bull EBITDA (US$M) = license GP + AMT-130 revenue - COGS 963.0 - cash opex 824.0 (core Q2 2026 run rate x4 +3%/yr, US commercial fixed $90M, $1.5M per active center, 5% variable, UK $15M from FY28) [S] (D2, unverified)
- **S39** bull EBITDA (US$M) = license GP + AMT-130 revenue - COGS 856.0 - cash opex 818.5 (core Q2 2026 run rate x4 +3%/yr, US commercial fixed $90M, $1.5M per active center, 5% variable, UK $15M from FY28) [S] (D2, unverified)
- **S40** bull EBITDA (US$M) = license GP + AMT-130 revenue - COGS 724.4 - cash opex 801.0 (core Q2 2026 run rate x4 +3%/yr, US commercial fixed $90M, $1.5M per active center, 5% variable, UK $15M from FY28) [S] (D2, unverified)
- **S41** bull EBITDA (US$M) = license GP + AMT-130 revenue - COGS 678.4 - cash opex 788.7 (core Q2 2026 run rate x4 +3%/yr, US commercial fixed $90M, $1.5M per active center, 5% variable, UK $15M from FY28) [S] (D2, unverified)
- **S42** Average of me and FY33 gross margin (Finagle default rule); no positive-EBITDA history to set it against [S] (D2, unverified)
- **S43** FY39 growth: royalty 53.8 (+3%) + AMT-130 at the FY33 steady-state flow and price, vs FY33 revenue [S] (D8, unverified)
- **S44** No AMT-130 US revenue: bear case, AMT-130 not approved. (D5, unverified)
- **S45** No AMT-130 UK revenue in bear (D4, unverified)
- **S47** bear: license gross profit - cash opex 200 (restructured opex 170 + confirmatory study 30) (US$M) [S] (D3, unverified)

- **S48** bear: license gross profit - cash opex 195 (restructured opex 150 + confirmatory study 45) (US$M) [S] (D3, unverified)
- **S49** bear: license gross profit - cash opex 172 (restructured opex 140 + confirmatory study 32) (US$M) [S] (D3, unverified)
- **S50** bear: license gross profit - cash opex 157 (restructured opex 140 + confirmatory study 17) (US$M) [S] (D3, unverified)
- **S51** bear: license gross profit - cash opex 145 (restructured opex 140 + confirmatory study 5) (US$M) [S] (D3, unverified)
- **S52** bear: license gross profit - cash opex 140 (restructured opex 140 + confirmatory study 0) (US$M) [S] (D3, unverified)
- **S53** bear: license gross profit - cash opex 140 (restructured opex 140 + confirmatory study 0) (US$M) [S] (D3, unverified)
- **S54** Bear: opex fixed after restructuring; an incremental royalty dollar drops through at the license gross margin [S] (D2, unverified)
- **S55** FY39 growth: Hemgenix royalty only, +3% [S] (D8, unverified)
- **S401** Revenue recognised at treatment (point in time), usual practice for one-time gene therapies; not a uniQure policy disclosure [S]
- **S402** Q3 2026 results date 2026-11-04 (not reopened this revision) [S]
- **S403** Judgment (untagged in the forecast file): raise sizes and SBC percentage
- **S601** Leave rate m = 0.62 a year (progression 0.60, death 0.02): analyst choice for internal consistency with the 6,000
- **S602** HD incidence 1.22 per 100k, US population 340M, 90% early-manifest share, population growth 0.5% a year
- **S603** Referral, willingness and coverage at 90% each, so P = 0.729
- **S604** CLPT AMT-130 capacity ramp 330 / 2,500 / 3,000 cap; centre arithmetic is analyst judgment
- **S605** Net price $2.0M per patient
- **S901** Aspen Phase 3 size assumed at about 100 patients, in line with the AskBio and BlueRock trials (D908, D909); the 424B4 does not state the number of patients [S]

- **S902** About $10M a year of the Q2 2026 AMT-130 external run rate (~$25M a year, D904) is redeployed to the confirmatory study, so the separate line = gross cost − $10M a year, floored at zero [S] **Calculations**

- **C2** base US entrants to the early-manifest pool 2027 = 1.22/100k x 340M x share 0.90 x filter 1.0045 x 1.005 = 3769/yr (+0.5%/yr); filter holds the steady pool at the 6,000 start pool (reference run case A) (D11, unverified)

- **C3** bull US entrants to the early-manifest pool 2027 = 1.59/100k x 340M x share 0.95 x filter 1.0000 x 1.005 = 5161/yr (+0.5%/yr); unfiltered (D11, unverified)
- **C8** FY26 consensus EUR x fx; all royalty/license, no AMT-130 sales before approval (D6, unverified)
- **C11** base AMT-130 US (US$M) = 330 patients treated (min of CLPT capacity 330/yr and pool demand, m = 0.62) x $2.00M net, recognised at treatment (D8, unverified)
- **C12** base AMT-130 US (US$M) = 2500 patients treated (min of CLPT capacity 2500/yr and pool demand, m = 0.62) x $2.00M net, recognised at treatment (D8, unverified)
- **C13** base AMT-130 US (US$M) = 3000 patients treated (min of CLPT capacity 3000/yr and pool demand, m = 0.62) x $2.00M net, recognised at treatment (D8, unverified)
- **C14** base AMT-130 US (US$M) = 2984 patients treated (min of CLPT capacity 3000/yr and pool demand, m = 0.62) x $1.80M net, recognised at treatment (D8, unverified)
- **C15** base AMT-130 US (US$M) = 2840 patients treated (min of CLPT capacity 3000/yr and pool demand, m = 0.62) x $1.80M net, recognised at treatment (D8, unverified)
- **C16** base AMT-130 US (US$M) = 2832 patients treated (min of CLPT capacity 3000/yr and pool demand, m = 0.62) x $1.80M net, recognised at treatment (D8, unverified)
- **C20** Sum of base lines (US$M) (D8, unverified)
- **C21** Sum of base lines (US$M) (D8, unverified)
- **C22** Sum of base lines (US$M) (D8, unverified)
- **C23** Sum of base lines (US$M) (D8, unverified)
- **C24** Sum of base lines (US$M) (D8, unverified)
- **C25** Sum of base lines (US$M) (D8, unverified)
- **C26** Sum of base lines (US$M) (D8, unverified)
- **C27** EBITDA = loss from operations -184,294 + D&A 12,641 + SBC 22,258 (2024) (D1, unverified)
- **C28** EBITDA = loss from operations -185,349 + D&A 14,759 + SBC 17,679 (2025) (D1, unverified)
- **C31** H1A EBITDA + H2 revenue x license GM - 2 x Q2 cash opex net of other items (47.771) [S run rate] (D3, unverified)
- **C32** (revenue - cost of license revenues 1,686)/revenue 2025 (D1, unverified)

- **C33** Revenue-weighted gross margin base (license GM; AMT-130 1 - COGS/revenue) (D2, unverified)
- **C34** Revenue-weighted gross margin base (license GM; AMT-130 1 - COGS/revenue) (D2, unverified)
- **C35** Revenue-weighted gross margin base (license GM; AMT-130 1 - COGS/revenue) (D2, unverified)
- **C36** Revenue-weighted gross margin base (license GM; AMT-130 1 - COGS/revenue) (D2, unverified)
- **C37** Revenue-weighted gross margin base (license GM; AMT-130 1 - COGS/revenue) (D2, unverified)
- **C38** Revenue-weighted gross margin base (license GM; AMT-130 1 - COGS/revenue) (D2, unverified)
- **C39** Revenue-weighted gross margin base (license GM; AMT-130 1 - COGS/revenue) (D2, unverified)
- **C40** Revenue-weighted gross margin base (license GM; AMT-130 1 - COGS/revenue) (D2, unverified)
- **C43** FY33E EBITDA / FY33E revenue (steady state: treated = pool demand, base; bull at capacity cap), base (D3, unverified)
- **C44** US centers implied at year end = capacity / (2/week x 52) (count), base FY26E (D32, unverified)
- **C45** US centers implied at year end = capacity / (2/week x 52) (count), base FY27E (D32, unverified)
- **C46** US centers implied at year end = capacity / (2/week x 52) (count), base FY28E (D32, unverified)
- **C47** US centers implied at year end = capacity / (2/week x 52) (count), base FY29E (D32, unverified)
- **C48** US centers implied at year end = capacity / (2/week x 52) (count), base FY30E (D32, unverified)
- **C49** US centers implied at year end = capacity / (2/week x 52) (count), base FY31E (D32, unverified)
- **C50** US centers implied at year end = capacity / (2/week x 52) (count), base FY32E (D32, unverified)
- **C51** US centers implied at year end = capacity / (2/week x 52) (count), base FY33E (D32, unverified)
- **C52** US treatment capacity = CLPT AMT-130 ramp (patients/yr), base FY26E (D32, unverified)
- **C53** US treatment capacity = CLPT AMT-130 ramp (patients/yr), base FY27E (D32, unverified)
- **C54** US treatment capacity = CLPT AMT-130 ramp (patients/yr), base FY28E (D32, unverified)
- **C55** US treatment capacity = CLPT AMT-130 ramp (patients/yr), base FY29E (D32, unverified)
- **C56** US treatment capacity = CLPT AMT-130 ramp (patients/yr), base FY30E (D32, unverified)
- **C57** US treatment capacity = CLPT AMT-130 ramp (patients/yr), base FY31E (D32, unverified)
- **C58** US treatment capacity = CLPT AMT-130 ramp (patients/yr), base FY32E (D32, unverified)
- **C59** US treatment capacity = CLPT AMT-130 ramp (patients/yr), base FY33E (D32, unverified)
- **C60** US entrants to the eligible pool (patients/yr), base FY26E (D11, unverified)
- **C61** US entrants to the eligible pool (patients/yr), base FY27E (D11, unverified)
- **C62** US entrants to the eligible pool (patients/yr), base FY28E (D11, unverified)
- **C63** US entrants to the eligible pool (patients/yr), base FY29E (D11, unverified)
- **C64** US entrants to the eligible pool (patients/yr), base FY30E (D11, unverified)
- **C65** US entrants to the eligible pool (patients/yr), base FY31E (D11, unverified)
- **C66** US entrants to the eligible pool (patients/yr), base FY32E (D11, unverified)
- **C67** US entrants to the eligible pool (patients/yr), base FY33E (D11, unverified)
- **C68** US pool demand = u x pool (patients/yr), base FY26E (D31, unverified)
- **C69** US pool demand = u x pool (patients/yr), base FY27E (D31, unverified)
- **C70** US pool demand = u x pool (patients/yr), base FY28E (D31, unverified)
- **C71** US pool demand = u x pool (patients/yr), base FY29E (D31, unverified)
- **C72** US pool demand = u x pool (patients/yr), base FY30E (D31, unverified)
- **C73** US pool demand = u x pool (patients/yr), base FY31E (D31, unverified)
- **C74** US pool demand = u x pool (patients/yr), base FY32E (D31, unverified)
- **C75** US pool demand = u x pool (patients/yr), base FY33E (D31, unverified)
- **C76** US untreated pool leavers = m x pool (patients/yr), base FY26E (D31, unverified)
- **C77** US untreated pool leavers = m x pool (patients/yr), base FY27E (D31, unverified)

- **C78** US untreated pool leavers = m x pool (patients/yr), base FY28E (D31, unverified)
- **C79** US untreated pool leavers = m x pool (patients/yr), base FY29E (D31, unverified)
- **C80** US untreated pool leavers = m x pool (patients/yr), base FY30E (D31, unverified)
- **C81** US untreated pool leavers = m x pool (patients/yr), base FY31E (D31, unverified)
- **C82** US untreated pool leavers = m x pool (patients/yr), base FY32E (D31, unverified)
- **C83** US untreated pool leavers = m x pool (patients/yr), base FY33E (D31, unverified)
- **C84** US eligible pool at year end = start 6,000 + entrants - leavers - treated (patients), base FY26E (D10, unverified)
- **C85** US eligible pool at year end = start 6,000 + entrants - leavers - treated (patients), base FY27E (D10, unverified)
- **C86** US eligible pool at year end = start 6,000 + entrants - leavers - treated (patients), base FY28E (D10, unverified)
- **C87** US eligible pool at year end = start 6,000 + entrants - leavers - treated (patients), base FY29E (D10, unverified)
- **C88** US eligible pool at year end = start 6,000 + entrants - leavers - treated (patients), base FY30E (D10, unverified)
- **C89** US eligible pool at year end = start 6,000 + entrants - leavers - treated (patients), base FY31E (D10, unverified)
- **C90** US eligible pool at year end = start 6,000 + entrants - leavers - treated (patients), base FY32E (D10, unverified)
- **C91** US eligible pool at year end = start 6,000 + entrants - leavers - treated (patients), base FY33E (D10, unverified)
- **C92** US patients treated = min(CLPT capacity, pool demand) (patients/yr), base FY26E (D31, unverified)
- **C93** US patients treated = min(CLPT capacity, pool demand) (patients/yr), base FY27E (D31, unverified)
- **C94** US patients treated = min(CLPT capacity, pool demand) (patients/yr), base FY28E (D31, unverified)
- **C95** US patients treated = min(CLPT capacity, pool demand) (patients/yr), base FY29E (D31, unverified)
- **C96** US patients treated = min(CLPT capacity, pool demand) (patients/yr), base FY30E (D31, unverified)
- **C97** US patients treated = min(CLPT capacity, pool demand) (patients/yr), base FY31E (D31, unverified)
- **C98** US patients treated = min(CLPT capacity, pool demand) (patients/yr), base FY32E (D31, unverified)
- **C99** US patients treated = min(CLPT capacity, pool demand) (patients/yr), base FY33E (D31, unverified)
- **C100** US cumulative patients treated (patients), base FY26E (D8, unverified)
- **C101** US cumulative patients treated (patients), base FY27E (D8, unverified)
- **C102** US cumulative patients treated (patients), base FY28E (D8, unverified)
- **C103** US cumulative patients treated (patients), base FY29E (D8, unverified)
- **C104** US cumulative patients treated (patients), base FY30E (D8, unverified)
- **C105** US cumulative patients treated (patients), base FY31E (D8, unverified)
- **C106** US cumulative patients treated (patients), base FY32E (D8, unverified)
- **C107** US cumulative patients treated (patients), base FY33E (D8, unverified)
- **C108** bull AMT-130 US (US$M) = 880 patients treated (min of CLPT capacity 880/yr and pool demand, m = 0.62) x $2.40M net, recognised at treatment (D8, unverified)
- **C109** bull AMT-130 US (US$M) = 4500 patients treated (min of CLPT capacity 4500/yr and pool demand, m = 0.62) x $2.16M net, recognised at treatment (D8, unverified)
- **C110** bull AMT-130 US (US$M) = 3500 patients treated (min of CLPT capacity 3500/yr and pool demand, m = 0.62) x $2.16M net, recognised at treatment (D8, unverified)
- **C111** bull AMT-130 UK (US$M) = 117 UK patients treated x $1.5M net (D13, unverified)
- **C112** bull AMT-130 UK (US$M) = 663 UK patients treated x $1.5M net (D13, unverified)
- **C113** bull AMT-130 UK (US$M) = 780 UK patients treated x $1.5M net (D13, unverified)
- **C114** bull AMT-130 UK (US$M) = 525 UK patients treated x $1.5M net (D13, unverified)
- **C115** bull AMT-130 UK (US$M) = 269 UK patients treated x $1.5M net (D13, unverified)
- **C117** Sum of bull lines (US$M) (D8, unverified)
- **C118** Sum of bull lines (US$M) (D8, unverified)
- **C119** Sum of bull lines (US$M) (D8, unverified)
- **C120** Sum of bull lines (US$M) (D8, unverified)
- **C121** Sum of bull lines (US$M) (D8, unverified)

- **C122** Sum of bull lines (US$M) (D8, unverified)
- **C123** Sum of bull lines (US$M) (D8, unverified)
- **C124** Revenue-weighted gross margin bull (license GM; AMT-130 1 - COGS/revenue) (D2, unverified)
- **C125** Revenue-weighted gross margin bull (license GM; AMT-130 1 - COGS/revenue) (D2, unverified)
- **C126** Revenue-weighted gross margin bull (license GM; AMT-130 1 - COGS/revenue) (D2, unverified)
- **C127** Revenue-weighted gross margin bull (license GM; AMT-130 1 - COGS/revenue) (D2, unverified)
- **C128** Revenue-weighted gross margin bull (license GM; AMT-130 1 - COGS/revenue) (D2, unverified)
- **C129** Revenue-weighted gross margin bull (license GM; AMT-130 1 - COGS/revenue) (D2, unverified)
- **C130** Revenue-weighted gross margin bull (license GM; AMT-130 1 - COGS/revenue) (D2, unverified)
- **C131** Revenue-weighted gross margin bull (license GM; AMT-130 1 - COGS/revenue) (D2, unverified)
- **C134** FY33E EBITDA / FY33E revenue (steady state: treated = pool demand, base; bull at capacity cap), bull (D3, unverified)
- **C135** US centers implied at year end = capacity / (2/week x 52) (count), bull FY26E (D32, unverified)
- **C136** US centers implied at year end = capacity / (2/week x 52) (count), bull FY27E (D32, unverified)
- **C137** US centers implied at year end = capacity / (2/week x 52) (count), bull FY28E (D32, unverified)
- **C138** US centers implied at year end = capacity / (2/week x 52) (count), bull FY29E (D32, unverified)
- **C139** US centers implied at year end = capacity / (2/week x 52) (count), bull FY30E (D32, unverified)
- **C140** US centers implied at year end = capacity / (2/week x 52) (count), bull FY31E (D32, unverified)
- **C141** US centers implied at year end = capacity / (2/week x 52) (count), bull FY32E (D32, unverified)
- **C142** US centers implied at year end = capacity / (2/week x 52) (count), bull FY33E (D32, unverified)
- **C143** US treatment capacity = CLPT AMT-130 ramp (patients/yr), bull FY26E (D32, unverified)
- **C144** US treatment capacity = CLPT AMT-130 ramp (patients/yr), bull FY27E (D32, unverified)
- **C145** US treatment capacity = CLPT AMT-130 ramp (patients/yr), bull FY28E (D32, unverified)
- **C146** US treatment capacity = CLPT AMT-130 ramp (patients/yr), bull FY29E (D32, unverified)
- **C147** US treatment capacity = CLPT AMT-130 ramp (patients/yr), bull FY30E (D32, unverified)
- **C148** US treatment capacity = CLPT AMT-130 ramp (patients/yr), bull FY31E (D32, unverified)
- **C149** US treatment capacity = CLPT AMT-130 ramp (patients/yr), bull FY32E (D32, unverified)
- **C150** US treatment capacity = CLPT AMT-130 ramp (patients/yr), bull FY33E (D32, unverified)
- **C151** US entrants to the eligible pool (patients/yr), bull FY26E (D11, unverified)
- **C152** US entrants to the eligible pool (patients/yr), bull FY27E (D11, unverified)
- **C153** US entrants to the eligible pool (patients/yr), bull FY28E (D11, unverified)
- **C154** US entrants to the eligible pool (patients/yr), bull FY29E (D11, unverified)
- **C155** US entrants to the eligible pool (patients/yr), bull FY30E (D11, unverified)
- **C156** US entrants to the eligible pool (patients/yr), bull FY31E (D11, unverified)
- **C157** US entrants to the eligible pool (patients/yr), bull FY32E (D11, unverified)
- **C158** US entrants to the eligible pool (patients/yr), bull FY33E (D11, unverified)
- **C159** US pool demand = u x pool (patients/yr), bull FY26E (D31, unverified)
- **C160** US pool demand = u x pool (patients/yr), bull FY27E (D31, unverified)
- **C161** US pool demand = u x pool (patients/yr), bull FY28E (D31, unverified)
- **C162** US pool demand = u x pool (patients/yr), bull FY29E (D31, unverified)
- **C163** US pool demand = u x pool (patients/yr), bull FY30E (D31, unverified)
- **C164** US pool demand = u x pool (patients/yr), bull FY31E (D31, unverified)
- **C165** US pool demand = u x pool (patients/yr), bull FY32E (D31, unverified)
- **C166** US pool demand = u x pool (patients/yr), bull FY33E (D31, unverified)

- **C167** US untreated pool leavers = m x pool (patients/yr), bull FY26E (D31, unverified)
- **C168** US untreated pool leavers = m x pool (patients/yr), bull FY27E (D31, unverified)
- **C169** US untreated pool leavers = m x pool (patients/yr), bull FY28E (D31, unverified)
- **C170** US untreated pool leavers = m x pool (patients/yr), bull FY29E (D31, unverified)
- **C171** US untreated pool leavers = m x pool (patients/yr), bull FY30E (D31, unverified)
- **C172** US untreated pool leavers = m x pool (patients/yr), bull FY31E (D31, unverified)
- **C173** US untreated pool leavers = m x pool (patients/yr), bull FY32E (D31, unverified)
- **C174** US untreated pool leavers = m x pool (patients/yr), bull FY33E (D31, unverified)
- **C175** US eligible pool at year end = start 6,000 + entrants - leavers - treated (patients), bull FY26E (D10, unverified)
- **C176** US eligible pool at year end = start 6,000 + entrants - leavers - treated (patients), bull FY27E (D10, unverified)
- **C177** US eligible pool at year end = start 6,000 + entrants - leavers - treated (patients), bull FY28E (D10, unverified)
- **C178** US eligible pool at year end = start 6,000 + entrants - leavers - treated (patients), bull FY29E (D10, unverified)
- **C179** US eligible pool at year end = start 6,000 + entrants - leavers - treated (patients), bull FY30E (D10, unverified)
- **C180** US eligible pool at year end = start 6,000 + entrants - leavers - treated (patients), bull FY31E (D10, unverified)
- **C181** US eligible pool at year end = start 6,000 + entrants - leavers - treated (patients), bull FY32E (D10, unverified)
- **C182** US eligible pool at year end = start 6,000 + entrants - leavers - treated (patients), bull FY33E (D10, unverified)
- **C183** US patients treated = min(CLPT capacity, pool demand) (patients/yr), bull FY26E (D31, unverified)
- **C184** US patients treated = min(CLPT capacity, pool demand) (patients/yr), bull FY27E (D31, unverified)
- **C185** US patients treated = min(CLPT capacity, pool demand) (patients/yr), bull FY28E (D31, unverified)
- **C186** US patients treated = min(CLPT capacity, pool demand) (patients/yr), bull FY29E (D31, unverified)
- **C187** US patients treated = min(CLPT capacity, pool demand) (patients/yr), bull FY30E (D31, unverified)
- **C188** US patients treated = min(CLPT capacity, pool demand) (patients/yr), bull FY31E (D31, unverified)
- **C189** US patients treated = min(CLPT capacity, pool demand) (patients/yr), bull FY32E (D31, unverified)
- **C190** US patients treated = min(CLPT capacity, pool demand) (patients/yr), bull FY33E (D31, unverified)
- **C191** US cumulative patients treated (patients), bull FY26E (D8, unverified)
- **C192** US cumulative patients treated (patients), bull FY27E (D8, unverified)
- **C193** US cumulative patients treated (patients), bull FY28E (D8, unverified)
- **C194** US cumulative patients treated (patients), bull FY29E (D8, unverified)
- **C195** US cumulative patients treated (patients), bull FY30E (D8, unverified)
- **C196** US cumulative patients treated (patients), bull FY31E (D8, unverified)
- **C197** US cumulative patients treated (patients), bull FY32E (D8, unverified)
- **C198** US cumulative patients treated (patients), bull FY33E (D8, unverified)
- **C199** UK capacity (patients/yr), bull FY26E (D13, unverified)
- **C200** UK capacity (patients/yr), bull FY27E (D13, unverified)
- **C201** UK capacity (patients/yr), bull FY28E (D13, unverified)
- **C202** UK capacity (patients/yr), bull FY29E (D13, unverified)
- **C203** UK capacity (patients/yr), bull FY30E (D13, unverified)
- **C204** UK capacity (patients/yr), bull FY31E (D13, unverified)
- **C205** UK capacity (patients/yr), bull FY32E (D13, unverified)
- **C206** UK capacity (patients/yr), bull FY33E (D13, unverified)
- **C207** UK patients treated (patients/yr), bull FY26E (D13, unverified)
- **C208** UK patients treated (patients/yr), bull FY27E (D13, unverified)
- **C209** UK patients treated (patients/yr), bull FY28E (D13, unverified)

- **C210** UK patients treated (patients/yr), bull FY29E (D13, unverified)
- **C211** UK patients treated (patients/yr), bull FY30E (D13, unverified)
- **C212** UK patients treated (patients/yr), bull FY31E (D13, unverified)
- **C213** UK patients treated (patients/yr), bull FY32E (D13, unverified)
- **C214** UK patients treated (patients/yr), bull FY33E (D13, unverified)
- **C215** UK eligible backlog at year end (patients), bull FY26E (D13, unverified)
- **C216** UK eligible backlog at year end (patients), bull FY27E (D13, unverified)
- **C217** UK eligible backlog at year end (patients), bull FY28E (D13, unverified)
- **C218** UK eligible backlog at year end (patients), bull FY29E (D13, unverified)
- **C219** UK eligible backlog at year end (patients), bull FY30E (D13, unverified)
- **C220** UK eligible backlog at year end (patients), bull FY31E (D13, unverified)
- **C221** UK eligible backlog at year end (patients), bull FY32E (D13, unverified)
- **C222** UK eligible backlog at year end (patients), bull FY33E (D13, unverified)
- **C223** Sum of bear lines (US$M) (D8, unverified)
- **C224** Sum of bear lines (US$M) (D8, unverified)
- **C225** Sum of bear lines (US$M) (D8, unverified)
- **C226** Sum of bear lines (US$M) (D8, unverified)
- **C227** Sum of bear lines (US$M) (D8, unverified)
- **C228** Sum of bear lines (US$M) (D8, unverified)
- **C229** Sum of bear lines (US$M) (D8, unverified)
- **C230** Revenue-weighted gross margin bear (license GM; AMT-130 1 - COGS/revenue) (D2, unverified)
- **C233** FY33E EBITDA / FY33E revenue (steady state: treated = pool demand, base; bull at capacity cap), bear (D3, unverified)
- **C236** FY27 consensus / base line build: NOT applied (P5 capacity instruction overrides consensus); far from 1, flagged (D6, unverified)
- **C401** Growth = revenue / prior revenue - 1; EBITDA margin = EBITDA / revenue; line share = line / total; from the forecast file
- **C504** new shares = raise / fetched price US$25.30, from D45
- **C508** bear raise / (fetched price x 68.216221M shares after the June 2026 offering) = 4,250 / 1,726, from D45
- **C509** value per share / price - 1, from D45
- **C510** leave rate = -ln(1 - 2/6) over ~1 year = ~0.41/yr, from A23
- **C511** 80.240 + 542.301 + 242.681 = 865.222; shares 62.529408 + 5.686813 = 68.216221
- **C601** 1.22e-5 x 340M x 0.9 = ~3,733 in 2026, ~3,750 to 3,770 in 2027 with growth; 3,733 / 0.625 = ~5,973
- **C602** Quarterly simulation, /workspace/out/AMT130_demand/scratch_m62/run16.py (filter factor 1.0045)
- **C603** Pool identity E - mS - T = gS with T = uS = PE gives u = P(m + g)/(1 - P), exact in any year; model ceiling u_d = 0.729 x (0.62 + 0.005)/(1 - 0.729) = 1.68 a year
- **C604** Bull ramp, scratch_m62/bull_chart.py: treated 880 / 4,498 / 3,073 / 2,786, then ~2,800 to 2,850
- **C605** Cohort tracking with the even-draw rule: 2,232 of the launch 6,000 treated
- **C606** 3,000 x $2.0M = $6.0B
- **C607** 1.6 years x ~1 point a year = ~1.6 points; a 4-year window gives m = ~0.27 and a pool of 3,750 / 0.27 = ~13,800
- **C608** scratch_m62/run25.py: unfiltered steady pool 9,218 at m = 0.40, filter factor 0.65
- **C609** -ln(1 - 1/3) = ~0.41 a year, so a stay of ~1 / 0.41 = ~2.4 years
- **C610** Annual terms from a copy of scratch_m62/run16.py case A (/workspace/out/QURE_run_v14/amt130_annual_terms.py; results amt130_annual_terms.json): sums of quarterly entrants, leavers and treated; P = T/E, u = T/S, g = end/S - 1, m_eff = L/S
- **C611** Identity check from amt130_annual_terms.json: |E - L - T - (end - S)| < 1e-9 and P(m_eff + g)/(1 - P) = u in every year FY2027 to FY2035
- **C612** UK entrants = 0.72e-5 x 69M x 1.005^2 x 0.9 = ~452 a year at launch; start pool = entrants / (0.62 + 0.005) = 723; long-run demand = 0.729 x entrants = 329; capacity year 1 = max(2 x 52, 329/3) = 110, year 2 = 219, year 3+ = 329; centres = capacity / 52

- **C613** UK quarterly simulation, /workspace/out/AMT130_demand/uk_dry_run/uk_pool.py (results uk_pool_results.json, run inc0.72_A_1pw); identity residual below 1e-12 in every year
- **C614** uk_pool_results.json runs inc0.72_B_1pw (start pool 2,250), inc0.48_A_1pw and inc0.72_A_2pw
- **C615** UK revenue = treated x $1.5M, e.g. 329 x 1.5 = 494
- **C702** EBITDA excluding the study = forecast EBITDA + 70 in FY2027E-FY2031E; base FY2028E EBITDA at cost c = forecast FY2028E EBITDA + 70 - c; from the forecast file

- **C703** Debt drawn = FCFE - FCF; check EBITDA - capex - SBC - interest - cash tax = FCF within 0.05 in every year shown, from D45 **Benchmarks (confirmatory-study cost)**

- **B901** Moore TJ et al., JAMA Intern Med 2018;178(11):1451–57: "median estimated cost of $19.0 million... $41 117... per patient". https://jamanetwork.com/journals/jamainternalmedicine/fullarticle/2702287

- **B902** Sertkaya A et al., Clinical Trials 2016 / HHS ASPE: "a Phase 3 study cost ranged from US$11.5 million (dermatology) to US$52.9 (pain and anesthesia)". https://aspe.hhs.gov/reports/examination-clinical-trial-costs-barriers-drug-development-0 (web page; no page numbers)
- **B903** Health Advances, "Orphan drug development: what are the real costs": "large costs per patient (~$2MM-~$5.7MM)... median cost for the largest trial... ~$100MM (range $42MM-$175MM)". https://www.healthadvances.com/insights/blog/orphan-drug-development-what-are-the-real-costs (web page; no page numbers)
- **B904** GEN: "cost of goods for rAAV products ranging from roughly $500,000 to $1 million per dose". https://www.genengnews.com/topics/bioprocessing/regeneron-eliminates-endonuclease-in-aav-processing/ (web page; no page numbers)

- **B905** Roland Berger: Hemgenix "vector manufacturing cost alone of one dose could be in the range of USD1-2 million". https://www.rolandberger.com/en/Insights/Publications/Cutting-the-cost-of-gene-therapy-manufacturing.html (web page; no page numbers) **Inferences (confirmatory-study cost)**

- **I1** Confirmatory-study cost, separate opex line FY27E-FY31E (central profile, US$M): 30/45/32/17/5; = gross study cost 40/55/42/27/15 less ~$10M/yr already in the Q2 2026 R&D run rate (floored at 0); same in bear, base and bull. Built from uniQure AMT-130 direct R&D history, Aspen Neuroscience 424B4 use of proceeds, AskBio/BlueRock trial sizes and published benchmarks; cost undisclosed by uniQure [I] (D12, unverified)

- **I901** AMT-130 direct R&D 2017–H1 2026: 3.9 + 5.9 + 4.1 + 6.9 + 10.5 + 19.8 + 13.0 + 18.0 + 42.5 + 14.3 = $138.9M, from D901–D904
- **I902** Phase I/II cost per treated patient: $138.9M less $19.4M BLA preparation and ~$10M pre-IND work = ~$110M; $110M / 57 patients = ~$1.9M, from D902–D904
- **I903** Per dosing event excluding vector: ($110M − ~$30M fixed overhead − 205 patient-years × $0.15M = ~$31M) / 57 = ~$0.85M
- **I904** Cash walk mid-2026 to end-2029: $811.9M cash + $100M Hercules tranche − 14 quarters × ($34.0M R&D + $17.4M SG&A) − launch allowance, no AMT-130 sales; leaves ~0 with a ~$150M study and ~−$70M with a ~$250M study
- **I905** Aspen total Phase 3 cost: $110M / (0.50 to 0.67) = ~$165–220M; / ~100 patients = $1.6–2.2M per randomised patient
- **I906** Aspen scaled to AMT-130: 133 treated × $1.0–1.35M + 67 controls × $0.25–0.40M + $30–40M fixed = ~$180–260M
- **I907** Vector cost per AMT-130 patient $0.05–0.30M: a local brain dose scaled down from $0.5–1.0M generic AAV [B904] and $1–2M Hemgenix [B905]; dose ratio unverified

- **I908** Bottom-up build: treated patients × (surgery and site $0.2–0.85M + vector $0.05–0.30M + follow-up) + controls × follow-up + $30–40M fixed = $84–275M, middle $150M; combined with I906 and K901 into gross bands $120M / $190M / $290M (FY26–FY32), phased by trial activity **CEO-simulation output (labelled simulation)**

- **K901** Matt Kapusta bot: a CEO-simulation bot that reasons from public filings in the manner of uniQure's chief executive; a simulation, not a company statement. Output: gross study cost $190M / $250M / $320M (low / base / high); base phasing FY27–FY31 $50M / $75M / $62M / $38M / $25M; $1.2–1.8M per treated patient, $0.3–0.5M per control, $15–30M start-up. No page or link (simulation output).

Not verified

- D33, A1, A2, A3, A4, A5, A6, A7, A13, A14, A16, A21, A22, A23, A25, A26, G1, G3, G4, G5, G6, G9, G12, G13, G14, E6, P8, P9, S2, S4, S5, S6, S8, S9, S10, S11, S13, S14, S15, S16, S17, S18, S19, S20, S21, S22, S23, S24, S25, S26, S27, S28, S29, S30, S31, S34, S35, S36, S37, S38, S39, S40, S41, S42, S43, S44, S45, S47, S48, S49, S50, S51, S52, S53, S54, S55,

C2, C3, C8, C11, C12, C13, C14, C15, C16, C20, C21, C22, C23, C24, C25, C26, C27, C28, C31, C32, C33, C34, C35, C36, C37, C38, C39, C40, C43, C44, C45, C46, C47, C48, C49, C50, C51, C52, C53, C54, C55, C56, C57, C58, C59, C60, C61, C62, C63, C64, C65, C66, C67, C68, C69, C70, C71, C72, C73, C74, C75, C76, C77, C78, C79, C80, C81, C82, C83, C84, C85, C86, C87, C88, C89, C90, C91, C92, C93, C94, C95, C96, C97, C98, C99, C100, C101, C102, C103, C104, C105, C106, C107, C108, C109, C110, C111, C112, C113, C114, C115, C117, C118, C119, C120, C121, C122, C123, C124, C125, C126, C127, C128, C129, C130, C131, C134, C135, C136, C137, C138, C139, C140, C141, C142, C143, C144, C145, C146, C147, C148, C149, C150, C151, C152, C153, C154, C155, C156, C157, C158, C159, C160, C161, C162, C163, C164, C165, C166, C167, C168, C169, C170, C171, C172, C173, C174, C175, C176, C177, C178, C179, C180, C181, C182, C183, C184, C185, C186, C187, C188, C189, C190, C191, C192, C193, C194, C195, C196, C197, C198, C199, C200, C201, C202, C203, C204, C205, C206, C207, C208, C209, C210, C211, C212, C213, C214, C215, C216, C217, C218, C219, C220, C221, C222, C223, C224, C225, C226, C227, C228, C229, C230, C233, C236, I1: page or link unverified as marked in the forecast file. The crossover quote (A23, D33) is from a search result; the full release was not reopened. The ClearPoint ramp (D32) is our own uncommitted forecast case.

Appendix C – Method

We follow the forecaster method. Reported periods stay as reported. FY2026E is consensus, because there is no guidance. Later years are a quarterly line build: a pool of eligible patients is refilled by incidence and drained by treatment and by patients leaving the treatable window; treatment is capped by a capacity ramp; patients × net price set AMT-130 revenue. FY2027E is not scaled to consensus. Analyst instructions are tagged P and override the default method. Valuation numbers come only from Finagle's result file. Tags: A actual, G company statement, E external estimate, C calculation, S assumption, P analyst instruction, D document; Section 10 also uses B benchmark, I inference and K CEO-simulation output.

Appendix D – AMT-130 US patient demand model

This appendix sets out the US treatable-pool model behind the AMT-130 patient volumes in Section 7. Tags follow Appendix B: D document, G company statement, E external estimate, S assumption, C calculation. Statements marked "judgment" are the analyst's view, not sourced facts.

D.1 Summary

The model uses a leave rate of m = 0.62 a year for the US treatable pool: progression out of eligibility of 0.60 a year plus deaths of 0.02 a year, for an average stay in the treatable window of about 1.6 years [S601]. This is the analyst's choice because it is the most internally consistent option: it reconciles disease incidence with uniQure's own launch figure without any extra assumption.

The arithmetic is simple. Huntington's disease incidence of 1.22 per 100,000 a year across a US population of 340 million, times a 90% early-manifest share, gives about 3,750 to 3,770 new patients entering the treatable window each year [S602][C601]. A pool that gains that many a year and loses 0.62 of itself a year, plus population growth g of 0.5%, settles at 3,750 / (0.62 + 0.005), or about 5,970 [C601]. That matches uniQure's "~6,000 Initial Treatable Patients at Launch" [G601] with no filter needed.

This stay is shorter than the published duration evidence (D.6), which points to roughly 2.3 to 3 years. So m = 0.62 is a consistency choice, not a finding from the clinical data (judgment).

On this basis, with the ClearPoint (CLPT) capacity ramp, treated patients rise to the 3,000-a-year cap in FY2029. They hold there through FY2030, then ease to about 2,840 a year as demand from the pool becomes the limit [C602].

D.2 What the 6,000 is

uniQure's September 2025 pivotal deck (slide 25) shows nested circles titled "Large anticipated total addressable US market at time of launch". They run from about 100,000 genetically identifiable people, to about 40,000 symptomatic, to about 20,000 diagnosed, to "~6,000* Initial Treatable Patients at Launch", with the footnote "*Management

Estimate" [G601]. The September 24, 2025 prospectus says the same in words: "We estimate there will be 6,000 initial treatable patients addressable by AMT-130 in the U.S. at commercial launch, assuming approval" [G602]. A GEN report of the call paraphrases management as calling it about 30% of the roughly 20,000 diagnosed and symptomatic patients [D601].

No filing or call transcript reviewed says uniQure aims to *treat* 6,000 at launch [D607]. The wording "addressable" and "at time of launch" describes a stock of patients who would qualify on day one, so the model treats the 6,000 as a starting pool, not a treatment target. The criteria that produce the roughly 30% (functional score, age, CAG repeat length, brain volume) have not been disclosed [D607].

D.3 Method

The model is a quarterly stock-and-flow pool [D605]. Each quarter the pool of treatable patients starts at some level, gains new entrants, loses leavers who progress past eligibility or die, and loses those who are treated: pool at end = pool at start + entrants − leavers − treated, with leavers = m × pool.

Derivation of the demand formula. Take any year. Let S be the pool at the start of the year, E the entrants during the year, m the leave rate, T the number treated, P = T/E the share of that year's entrants treated, u = T/S the treatment rate on the pool, and g the pool's actual growth rate over the year, so that the pool changes by gS. The pool accounting is an identity:

E − mS − T = gS.

Writing T = uS and rearranging gives E = (m + g + u)S. Since T = PE, it follows that uS = P(m + g + u)S, so u = P(m + g + u), and solving for u:

u = P(m + g) / (1 − P) [C603].

Nothing in this step assumes a steady state. The relationship holds exactly in every year for whatever P and g that year produces, and a different P implies a different g. Because the model runs quarterly, leavers each quarter are (m/4) × S_q, where S_q is the pool at the start of the quarter, and treated is T_q. The annual figures in Figure D1b are built from the sums of the quarterly flows and the pool at the start of the year; the annual leave rate is then the effective rate m_eff = (sum of quarterly leavers) / S, which is below 0.62 whenever the pool falls during the year [C610].

How the model uses it. The model sets a demand rate u_d = P*(m + g*)/(1 − P*), with P* = 0.729 and g* = 0.5% a year (population growth), so u_d = 0.729 × 0.625 / 0.271 = 1.68 a year [C603]. Each quarter, demand is the ceiling (u_d/4) × S_q, and treated = min(capacity, (u_d/4) × S_q). In capacity-limited years the realised P is therefore set by capacity and the realised g is whatever the accounting produces, which is negative while the launch pool drains. In demand-limited years the realised P tends to 0.729 and pool growth tends to g*. A rate u above 1 a year is not a contradiction: the pool is drained by leavers and treatment at the same time as it is refilled by entrants, and the quarterly steps mean no quarter treats more patients than are in the pool.

P is 0.729: the share of eligible patients who are referred to a treatment centre (90%), agree to treatment (90%) and have it paid for (90%), so 0.9 × 0.9 × 0.9 = 0.729 [S603]. About three in four eligible patients make it through to surgery. The 90% levels are model assumptions, not company figures.

D.4 Capacity

Capacity follows the CLPT AMT-130 ramp [S604]. Launch is assumed on 1 July 2027. Capacity is 330 patients in the FY2027 half year, 2,500 in FY2028, and a cap of 3,000 a year from FY2029.

The ramp is built from about 10 centres doing 1.5 cases a week for 22 weeks, then about 20 centres doing 2.5 a week for 50 weeks. These figures are judgment, drawn from the ClearPoint CEO's capacity comments on the Q2 2026 call [D606]. On the same arithmetic 4,500 a year is a ceiling, not a cap. uniQure itself says only that capacity depends on neurosurgeons and operating suites per hospital, and that more detail will come [G603].

A bull ramp of 880 / 4,500 / 3,500 run with the same method [C604] reaches about 4,500 treated in FY2028, which draws the pool down to about 2,100. Treated then falls to about 3,070 in FY2029 and settles near 2,800 a year. Faster early capacity therefore produces a hump at this short stay, because demand rather than capacity sets the long-run level.

D.5 Results at m = 0.62

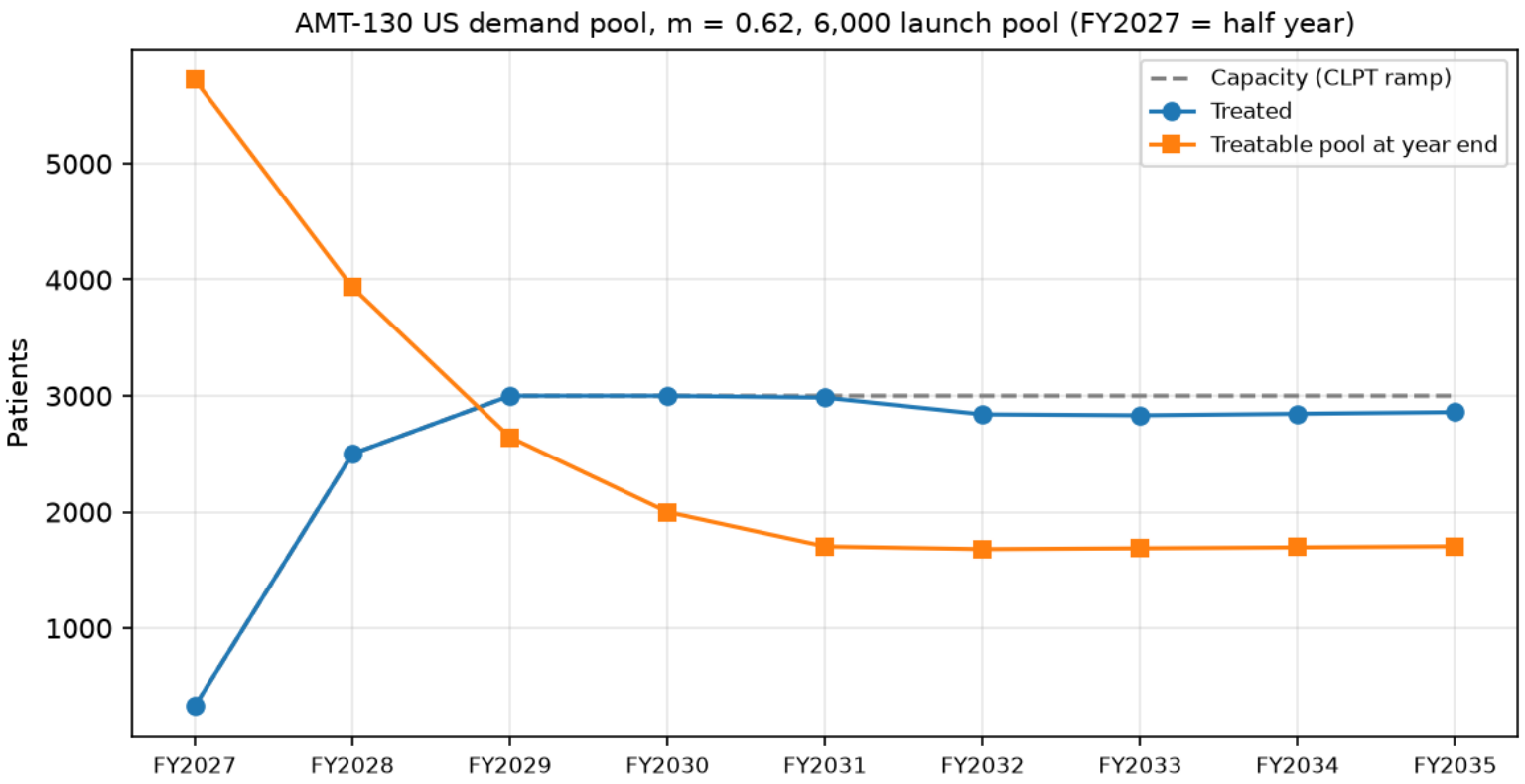


Figure D1 – Treated patients and pool, base case

Fiscal year	Capacity	Treated	Pool at year end	Cumulative treated
FY2027 (half year)	330	330	5,718	330
FY2028	2,500	2,500	3,933	2,830
FY2029	3,000	3,000	2,643	5,830
FY2030	3,000	3,000	2,000	8,830
FY2031	3,000	2,984	1,703	11,814
FY2032	3,000	2,840	1,680	14,654
FY2033	3,000	2,832	1,687	17,486
FY2035	3,000	2,859	1,704	23,190

Source: 6,000 starting pool, filter factor 1.0045, CLPT ramp [C602].

Figure D1b – Annual terms of the identity, base case

Fiscal year	Pool at start S	Entrants E	Leavers L	Treated T	Pool at end	m_eff = L/S	P = T/E	u = T/S	g	Binding limit
FY2027 (half year)[1]	6,000	1,884	1,836	330	5,718	0.306[1]	0.175[1]	0.055[1]	-4.7%[1]	Capacity
FY2028	5,718	3,788	3,072	2,500	3,933	0.537	0.660	0.44	-31.2%	Capacity
FY2029	3,933	3,807	2,097	3,000	2,643	0.533	0.788	0.76	-32.8%	Capacity
FY2030	2,643	3,826	1,468	3,000	2,000	0.556	0.784	1.14	-24.3%	Capacity
FY2031	2,000	3,845	1,157	2,984	1,703	0.579	0.776	1.49	-14.8%	Capacity Q1–Q3, demand Q4
FY2032	1,703	3,864	1,047	2,840	1,680	0.615	0.735	1.67	-1.4%	Demand
FY2033	1,680	3,883	1,044	2,832	1,687	0.622	0.729	1.69	0.4%	Demand
FY2034	1,687	3,903	1,049	2,845	1,696	0.622	0.729	1.69	0.5%	Demand
FY2035	1,696	3,922	1,054	2,859	1,704	0.622	0.729	1.69	0.5%	Demand

Source: quarterly simulation, m = 0.62, 6,000 launch pool, filter factor 1.0045, CLPT ramp [C602][C610]. E - L - T equals pool end - pool start to rounding in every year, and P(m_eff + g)/(1 - P) reproduces u in every year [C611]. The demand-ceiling rate u_d is 1.68 a year throughout [C603]. [1]FY2027 covers Q3–Q4 only (launch 1 July 2027); its values are raw half-year figures and are not annualised. Annual u is annual treated over the start-of-year pool, so it differs from the quarterly rate: in demand-limited years it comes out at about 1.69 rather than 1.68 because the pool is re-measured each quarter.

Reading the table. Through FY2030 capacity binds in every quarter, so capacity sets P and the accounting sets g. P rises from 0.18 in the half year to 0.66 in FY2028 and about 0.78 to 0.79 in FY2029 and FY2030, above 0.729, because the 3,000 slots are filled from a pool still larger than the long-run level. The pool shrinks fast, by 31% in FY2028 and 33% in FY2029, as leavers and treatment together exceed entrants. Demand first binds in the last quarter of FY2031 and in every quarter from FY2032; from then on referral, willingness and coverage set P. The pool reaches its low of about 1,680 at the end of FY2032 and turns up in FY2033, when g becomes positive (0.4%). Realised P reaches 0.729 in FY2033 and g reaches the 0.5% population growth rate in FY2034, so the long-run path is reached about six years after launch [C610].

Capacity is the limit through FY2030 and almost to the end of FY2031. After that, demand is the limit. Treated settles at about 2,840 a year, which is 0.729 × the roughly 3,900 entrants a year by then. Cumulative treated, counting new entrants, passes 6,000 early in FY2030.

Only about 2,230 of the 6,000 patients in the pool at launch are treated before they leave the window [C605], assuming each quarter's treatments are drawn evenly from launch patients and newer entrants. At m = 0.62 more than half of any group has progressed out within about a year, and capacity in the first 18 months is only 2,830, so most of the launch pool is lost to progression whatever capacity does.

While capacity binds, the treated path is the same for any m, because more patients are waiting than there are slots. What m changes is the leftover pool and how many launch patients are lost (judgment, consistent with the sensitivity runs in D.6).

As an illustration, at $2.0 million per patient, 3,000 patients a year is about $6.0 billion a year [S605][C606].

D.6 Duration evidence and sensitivity

The published evidence suggests patients stay in the early-manifest window longer than 1.6 years:

- A 2CARE analysis implies about 2.3 to 2.9 years [E601].
- A 2024 *Movement Disorders* paper implies 2 to 3 years [E602].
- An E070 dataset has a mean of 29 months, biased by censoring [E603].
- Marder et al. (2000) found a loss of about 0.97 TFC points a year from TFC 7 to 13, about 5 years from TFC 13 to below 9 [E604].
- The HDSA roadmap puts Stage I at roughly 0 to 8 years after motor diagnosis [E605].

At about one TFC point a year, a 1.6-year stay means a band only about 1.6 TFC points wide, for example 13 down to about 11.4, or about 11 down to 9.4 [C607]. That is a subset of the trial's TFC 9 to 13 window [D602]. The full window would mean a stay of about 4 years, m of about 0.27 and an unfiltered pool of about 13,800 [C607]. Reconciling that with 6,000 needs other eligibility filters (judgment).

Sensitivity: 2.5-year stay. At m = 0.40, holding the pool at 6,000 needs an extra filter that removes about 35% of patients [C608]. If the filter applies to both the launch stock and new entrants, treated peaks at about 2,900 in FY2029 and falls to about 1,850 a year. If it applies only to the launch stock, treated holds at 3,000 through FY2031 and settles near 2,850.

Crossover evidence. The US trial had 4 sham-surgery controls in the low-dose cohort and 6 in the high-dose cohort. After the blinded 12 months, 4 of the 6 high-dose controls crossed over to treatment; the other two "failed to meet the study's inclusion criteria" [G604][D603]. EHDN's November 2025 report says "four were eligible and accepted the offer" [D604]. Losing two of six in roughly a year implies a leave rate of about 0.41 a year, or a stay nearer 2.4 years [C609]. The sample is tiny, the failed criterion isn't stated, and the outcome for the low-dose controls was not found.

D.7 Limitations

- m = 0.62 (stay ~1.6 years) is shorter than the duration studies and the crossover evidence, which imply about 0.41 a year. A longer stay lowers long-run demand if the 6,000 reflects a filter on new entrants.
- The 6,000 is the company's addressable-at-launch management estimate, not a treatment target, and the criteria behind the roughly 30% are undisclosed.
- P = 0.729 is an assumption, not a company figure.
- The bull ramp produces a hump at this stay, because demand, not capacity, sets the long-run level.

Figure D2 – Verified vs assumed

Item	Status
~6,000 "Initial Treatable Patients at Launch", Management Estimate	Verified, slide read directly [G601]
Prospectus wording on 6,000	Quoted from secondary research, not reopened [G602]
~30% of ~20,000	Press paraphrase only [D601]
Crossover 4 of 6, 2 failed criteria	Search results and release snippets [G604][D603][D604]
Incidence 1.22/100k, 90% early-manifest share	Assumption [S602]
P = 0.729, $2.0M price, CLPT ramp	Assumptions [S603][S604][S605]
m = 0.62	Analyst's choice; shorter than the evidence [S601]
Duration studies E601 to E605	Snippets; papers not reopened

D.8 UK patient demand (illustrative)

This section applies the same pool model to the UK. The forecast's UK volumes (bull case only) come from a simple centre engine with a fixed backlog [S9]; the model here shows what an incidence-based pool gives instead. It is illustrative only and is not used in the forecast or the valuation.

Incidence. The UK rate is taken from Wexler et al. (2016), a study of UK primary-care records (the CPRD database): "the incidence of HD has remained constant between 1990 and 2010 with an overall rate of 7.2 (95% CI 6.5 to 7.9) per million person-years", which is 0.72 per 100,000 [D608]. It counts 393 new diagnoses recorded in general-practice records over 1990–2010, is not age-standardised, and its denominator is adults aged 21 and over. Only the abstract and methods were read. The bull case's UK assumption uses 0.48 per 100,000, which is the worldwide pooled rate of Pringsheim et al. (2012), not a UK figure [A16][S9]. The UK rate is about 40% below the US 1.22 used in D.1, although the US figure is from insurance claims, all ages and age-standardised, so part of the gap may reflect case counting rather than disease frequency (judgment).

Method. The structure and code logic are the same as the US model in D.3: a quarterly pool, entrants E = incidence × population × 0.90 early-manifest share, leavers (m/4) × pool, a demand ceiling u_d = P*(m + g*)/(1 − P*) = 1.68 a year, and treated each quarter = min(capacity, (u_d/4) × pool, pool). Capacity starts at two treating centres and ramps to long-run demand in thirds: in the first launch year (Q3 2028 to Q2 2029) capacity is the higher of two centres' throughput and one third of long-run demand; in the second year two thirds; from the third year all of it. Long-run demand is 0.729 × launch-year entrants [C612].

Figure D3 – UK model inputs

Input	Value	Type
UK population	69M, growing 0.5% a year	Assumption, as in the bull case's UK assumption [S9]
Incidence	0.72 per 100,000 (adults 21+)	Published study [D608]
Early-manifest share of entrants	90%	Assumption, as for the US [S602]
Leave rate m	0.62 a year (progression 0.60, death 0.02)	Judgment, as for the US [S601]
Treated share P*	0.729	Assumption, as for the US [S603]
Launch	Q3 2028	Assumption, as in the bull case's UK assumption [S9]
Net price	$1.5M per patient	Assumption, from the bull case's UK assumption [S9]
Throughput	1 procedure per centre per week (~52 a year)	Assumption, as in the bull case's UK assumption [S9]
Capacity	Two centres at launch, then 1/3, 2/3 and 3/3 of long-run demand	Analyst rule [C612]

Source: as tagged.

Starting pool. No company figure exists for the UK. The model starts from the incidence-implied pool E/(m + g) = 723 patients at launch, the same construction that reproduces uniQure's ~6,000 for the US [C612]. The bull case's UK assumption uses a backlog of 2,250 (30% × 7,500) [S9], about three times the incidence-implied pool, with no UK source. Starting from 2,250 instead leaves treated patients unchanged in every year, because capacity binds throughout; it only leaves a larger untreated pool that mostly ages out (1,764 at the end of FY2028 against 674) [C614].

Results. Long-run demand is 329 patients a year. Two centres at one procedure a week give 104 a year, slightly below one third of long-run demand (110), so the one-third level sets first-year capacity and the two-centre start never binds. Centres implied are 2.1, 4.2 and 6.3 in launch years one, two and three onward [C612]. Figure D4 reads like Figure D1b: each row is a year, with the pool at the start, the flows during the year and the realised terms of the identity in D.3.

Figure D4 – UK pool model, incidence 0.72, incidence-implied start pool, 1 procedure per centre per week

Fiscal year	Pool at start S	Entrants E	Leavers L	Treated T	Pool at end	P = T/E	u = T/S	g	Binding limit	Centres implied	Revenue at $1.5M (US$M)	Bull case UK: treated / revenue (US$M)
FY2028 (half year)[1]	723	226	220	55	674	0.24[1]	0.08[1]	-6.8%[1]	Capacity	2.1	82	117 / 176
FY2029	674	454	398	165	565	0.36	0.24	-16.1%	Capacity	3.2	247	663 / 994
FY2030	565	456	322	274	424	0.60	0.49	-24.9%	Capacity	5.3	412	780 / 1,170
FY2031	424	458	235	329	319	0.72	0.78	-25.0%	Capacity	6.3	494	780 / 1,170
FY2032	319	461	184	329	266	0.71	1.03	-16.4%	Capacity	6.3	494	524 / 787
FY2033	266	463	159	329	242	0.71	1.24	-9.3%	Capacity	6.3	494	269 / 404
FY2034	242	465	147	329	231	0.71	1.36	-4.5%	Capacity	6.3	494	–
FY2035	231	468	142	329	227	0.70	1.43	-1.6%	Capacity	6.3	494	–

Source: UK quarterly simulation [C613]; revenue = treated × $1.5M, illustrative only [C615]; bull case UK volumes and revenue from Appendix A [C209]–[C214][C111]–[C115]. E - L - T equals pool end - pool start to rounding in every year [C613]. [1]FY2028 covers Q3–Q4 only (launch 1 July 2028); its values are raw half-year figures and are not annualised, except centres implied, which is annualised capacity over per-centre throughput.

What this means. Capacity binds in every year. Treated rises to 329 a year from FY2031 and holds there, about 42% of the bull case's UK peak of 780; the pool falls from 723 to about 227 by FY2035. Because the capacity rule fixes the long-run level at launch-year demand while entrants grow 0.5% a year, realised P drifts down to about 0.70 rather than settling at 0.729. The bull case's UK assumption treats more because it starts from a 2,250 backlog and lets untreated patients leave at only 10% a year [S9].

Sensitivities [C614].

- **Incidence 0.48 per 100,000** (the worldwide pooled rate): start pool 482, long-run demand 219 a year; at one procedure a week the two-centre start (104) exceeds one third of demand (73) and sets first-year capacity; centres implied 2.0, 2.8 and 4.2.
- **Two procedures per centre per week** (incidence 0.72): two centres give 208 a year, above one third of long-run demand and close to two thirds (219); centres implied 2.0, 2.1 and 3.2. Long-run treated is unchanged at 329 a year.

Main caveat. The Wexler rate is per adult aged 21 and over, but it is applied here to the total UK population of 69M. That overstates entrants, and so long-run demand, by roughly a quarter to a third (judgment); the model does not correct for it, because no UK adult population share has been sourced.

Figure D5 – UK model: verified vs assumed

Item	Status
Incidence 0.72 per 100,000, adults 21+	Published study; abstract and methods read, full paper not reopened [D608]
Pringsheim 0.48 as a worldwide pooled rate	Forecast source entry, not reopened [A16]
UK population 69M	Taken from the bull case's UK assumption, not checked [S9]
90% early-manifest share, P* = 0.729	Assumptions [S602][S603]
m = 0.62	Judgment [S601]
$1.5M price, Q3 2028 launch, 1 procedure per centre per week	Assumptions from the bull case's UK assumption [S9]
Capacity ramp in thirds from a two-centre start	Analyst rule [C612]